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UNITED NATIONS DEVELOPMENT PROGRAMME

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PROJECT DOCUMENT
MOZAMBIQUE

Project Title: Health System Strengthening

Project Number: 00135528

Implementing Partner: UNDP

Start Date: 1 April 2021 **End Date:** 31 December 2022 **PAC Meeting date:** _____

Brief Description

This Project document is prepared to reflect the details of UNDP support as an Implementing Partner to the Government of Mozambique through the Ministry of Health (MoH), within the newly approved 3-year (2021-2023) Global Fund HIV Grant to Mozambique between MoH and the Global Fund (MOZ-H-MOH). The Health System Strengthening interventions align with the UNDP signature solution on poverty as it contributes to improve access to healthcare. UNDP Mozambique through a Financing Agreement (FA) modality will continue to work with the MoH, the Principal Recipient (PR), along with Central Medical Stores (CMAM), National Health Institute (INS) and Department of Infrastructure and Hospital Equipment (DIEH).

The Project is supporting the Government of Mozambique, through the Ministry of Health, in implementing the National Strategies towards achieving:

1. the mission of the National Strategic Plan for Pharmaceutical Logistics and its future integration with the Extended Vaccination Programme (PAV). The Project works to support the MoH in implementing National Logistics Pharmaceutical Plan (PELF) for establishing Medical Warehouses fully equipped including with Incinerators in strategic geographical areas that will ensure medical and non-medical products stocks available in quantity and quality necessary to be distributed through optimized and safer routes to reach the last mile and be accessible to all citizens at all times.
2. the mission of National Emergency Plan for preparedness and response to the COVID-19 pandemic, through strengthening laboratory capacity for decentralized testing, ensuring adequate and equipped health infrastructure, including through rehabilitation and installation of prefabricated modules. This intervention increases the diagnostic efficiency at sub-national level, aiming at providing citizens with their right to access quality services of treatment and care in a timely manner.

The long-term objective of the Project is to strengthen the national health system by reinforcing the medical supply chain system management and strengthening decentralized laboratory capacity.

This systemic approach is facilitating the current decentralization process of the health system promoted by the government which aims for better and timely delivery of medicines, treatments and healthcare to people at local level. A key strategy of the Project is implementation at decentralized levels to reach vulnerable populations and to decrease geographical inequalities – thereby contributing to the strategic objectives the National Strategic Health Plan (PESS).

The Project will be implemented by UNDP through Direct Implementation Modality (DIM), while ensuring a participatory approach that involves national counterparts, local authorities and representatives of main stakeholders and harnessing the existing strong collaboration with the Ministry of Health throughout all phases of implementation.

Contributing Outcome (UNDAF/CPD, RPD or GPD): UNDAF Outcome 8/ CPD Outcome 68 - All people benefit from democratic and transparent governance institutions and systems that guarantee peace consolidation, human rights and equitable service delivery. **Indicative Output(s) with gender marker:** Output 1: Improved safety, security and storage conditions of medicines, vaccines and other health products at sub-national level – GEN1 Output 2: Improved systems for adequate waste management – GEN1 Output 3: Strengthened laboratory capacity for decentralized COVID-19 testing – GEN1

Total resources required:		**$ 9,449,148.61**
Total resources allocated:		
	UNDP TRAC:	N/A
	Donor: **Global Fund through grant agreement MOZ-H-MOH**	**$9,449,148.61**
	Government:	N/A
	In-Kind:	N/A
Unfunded:	$0	

Government	**UNDP**
Permanent Secretary of the Ministry of Health Grilo da Silva Lubrino Print Name: Dr. Grilo da Silva Lubrino	Resident Representative of UNDP Print Name: Ms. Narjess Saidane
Date: 14.07.2021	Date: 08-Jul-2021

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ABBREVIATIONS

AGYW	Adolescent girls and young women
CO	Country Office
CSO	Civil Society Organizations
CPAP	Country Programme Action Plan
DHS	Demographic Health Survey
DHIS	District Health Information System
DIM	Direct Implementation Modality
FSW	Female Sex Workers
GDP	Gross Domestic Product
GFATM	The Global Fund
HSS	Health Systems Strengthening
IMASIDA	Inquérito de Indicadores de Imunização, Malária e HIV/SIDA em Moçambique
KP	Key Populations
M&E	Monitory and Evaluation
MoH	Ministry of Health
MSM	Men who have sex with men
OIG	Office of the Inspector General
PAV	Programa Alargado de Vacinação (Extended Vaccination Programme)
PES	Plano Económico e Social
PESS	Plano Estratégico do Sector da Saúde (Health Sector Strategic Plan)
PLHIV	People Living with HIV
PMU	Programme Management Unit
PPP	Purchasing Power Parity
PWID	People Who Inject Drugs
RRF	Results and Resources Framework
UN	United Nations
UNDAF	United Nations Development Assistance Framework
UNDP	United Nations Development Programme
UNICEF	United Nations Children's Fund
WHO	World Health Organization
WB	World Bank

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I. DEVELOPMENT CHALLENGE

The Republic of Mozambique has an estimated total population of 31,250,000 million (WB, 2019) with two-thirds of the population below 24 years of age (UN Population Fund, 2019).

Mozambique is a low-income economy. With current real gross domestic product (GDP) per capita estimated at under 590 USD (WB, 2019), the country is one of the poorest in the world. In terms of GDP per capita based on purchasing power parity, due to the relatively higher price for goods and services, the situation is even more alarming (WB 2019). The majority of the population (67%, WB 2019) lives in rural areas dedicated to agriculture. The urban-rural divide is often reflected in disparities in the access to and quality of basic social services, including health services. An inequality between Southern and Northern provinces also exists, both in terms of relative poverty and access to services, with Southern provinces having reached higher levels. Mozambique is ranked 180th from 189 on the Human Development Index.

Due to the effects of the 2016 hidden debt crisis, the real GDP growth decelerated to 3.7% in 2017, down from 3.8% in 2016 and well below the 7% GDP growth achieved on average between 2011 and 2015 (WB). The debt levels became unsustainably high. The low fiscal income together with the existing debts is making it challenging for the Government to increase expenditures in health and other sectors. The main macroeconomic challenges are to improve economic governance and transparency, as well as to create conditions for a sustained job growth.

The population growth is 2.9% (WB 2019) which is among the highest in the world. The population is expected to grow by 10 million in the next 10 years, reaching 65 million by 2050 (UN Population Fund, 2019 estimation). This population growth and shifts in the demographic structure of Mozambiquan population creates further pressure for government to meet its commitments for maternal and child health, education and creation of sufficient job growth.

Health profile

In 2019, Mozambique had an estimated 2.2 million people living with HIV (PLHIV), including 150,000 children. Prevalence varies significantly by gender, age and geography. Women face a disproportionate HIV burden throughout the life cycle, but the disparity is most pronounced among adolescent girls aged 15-19 years, whose HIV prevalence is 4.3 times greater than their male peers (6.5% vs. 1.5%).

Adult HIV incidence has fallen from 1.5% in 2000 to 0.9% in 2019, as a result of effective treatment and prevention programmes implemented by government and partners. Yet the total number of new infections remain high at 130,000 new infections/year in 2019. The largest number of new infections - about 45,000/year - occurs among young women (never married females, median age 18 years). This is followed by older men (previously married males, median age 32), Female Sex Workers (FSW), and previously married females (median age 38). These four populations account for about 81% of all new infections. FSWs and Men who have sex with men (MSM) make up a very small proportion of Mozambique's population (1.08% of the adult female population, and 0.8% of the adult male population, respectively) yet these two groups are estimated to account for 11% of new infections. These distinct sub epidemics require targeted responses. The National HIV Strategy 2015-2020 (PEN IV) defines key populations (KP) as FSW, MSM, People Who Inject Drugs (PWID) and prisoners. The plan also considers adolescent girls and young women (AGYW) aged 15-24 years, mobile and migrant workers (mine workers and truck drivers) and sero-discordant couples as vulnerable populations.

Mozambique has implemented rapid scale up of ART, guided by a National Acceleration Plan which resulted in the number of people on ART rising from 308,000 in 2012 to 1,338,000 in 2019. In addition, the country has been rolling out the "Test and Treat" policy since 2016. This together with scale up of other HIV prevention interventions has led to a reduction in the number of AIDS-related deaths, from a peak of 71,756 in 2006, to 50,587 in 2019. However, retention in care remains a major challenge, being at 67% at 12 months after enrolment in ART for adults in 2019.

Mozambique is also one of the highest burden countries for tuberculosis with a yearly incidence estimated at 550/100,000 inhabitants (WHO 2019). Mozambique´s population also suffers from high incidence rates of malaria, neglected tropical diseases and a growing burden of non-communicable diseases.

Thanks to targeted interventions, maternal mortality was lowered from 410 deaths/ 100,000 births in 2010 to an estimated 290 deaths/ 100,000 births in 2017 (WHO). While child under 5 mortality was also significantly reduced in the past 20 years, 7% of children still die before the age of five; 46% of children suffer malnutrition, one of the highest rates worldwide, exacerbating fatal outcomes for highly prevalent infectious diseases such as malaria, pneumonia and diarrhea (UNICEF).

Access to Health Services

Mozambique has 1,780 public health facilities reported into the DHIS 2 system (DHIS 2, August 2020). This is an increase from 1,643 public health facilities reported in 2018. The SARA-2018 study on the quality of health services stipulated that only 14% of facilities had six basic infrastructure conditions available (electricity, access to improved source of water, examination room offering privacy, improved toilets, means of and access to communication, means of transport). Currently there are 63 public hospitals, far below the government´s target of one hospital for each of Mozambique´s 157 districts.

Gender

Mozambique remains 142nd out of 164 countries on UNDP´s Gender Equality Index. Several gender aspects embody a significant development challenge. Strongly gendered cultural norms impose low decision-making power on women in questions affecting their every-day life and movements outside of the home, including their access to health services. Seventy percent of women have not finished primary education (DHS 2016). The low schooling among women impairs the ability to make health choices for themselves and their children. More than 42 percent of young women and girls experience some form of gender-based violence and 53% of girls get married before the age of 18, the minimum legal age for marriage. Also, 40% of women first give birth before the age of 18 (UNICEF 2020). The combined challenge of high morbimortality and limited access to health services, including hospital services, affects women. The HIV epidemic in the country affects women more than men, and pregnancies, deliveries and maternal health represent the largest percentage of hospital occupancy.

COVID-19 response

Mozambique has been exposed to fragilizing social economic factors prior to the COVID-19 crisis. The cyclones Idai and Kenneth created emergency situations in 2019 that are ongoing. The insurgency in Cabo Delgado continues as a complex crisis that Mozambique is trying to contain.

These development challenges described above further complicate and handicap an effective COVID-19 response. On one hand, the COVID-19 emergency response in Mozambique was quick and epidemiologically successful, notwithstanding a border with and population and trade flows to and from South Africa, the most affected country on the African continent. Regarding access, in practice some hospitals have been earmarked entirely for COVID-19 patients and closed for other treatments.

The increase of Laboratory capacity for decentralized COVID-19 testing continues to be a priority, through establishment of adequate and equipped infrastructure. Additionally, it has been verified a pressure on the hospital system, with the number of hospital beds in Mozambique at 0.5 beds/1000 population (SARA 2018), the country can ill afford the diversion of hospital services to yet another deadly disease. Only a fraction of the hospital beds has access to oxygen supply, important for treatment of serious COVID-19 cases. Conditions for hospitalizations in separate wards with independent work circuits and dedicated personnel, that would minimalize the risk of transmission for patients and health staff, often cannot be provided.

The Ministry of Health of Mozambique together with health partners and donors is addressing the emergency needs, that keep evolving as per the pandemic context.

II. STRATEGY

The Project "Health System Strengthening" will support the Government's efforts towards the achievement of the Agenda 2063 of the African Union - Objective 3: Citizens are healthy, well-nourished and have long life spans of the Aspiration 1: A Prosperous Africa based on Inclusive Growth and Sustainable Development.

The Project also contributes for the Agenda 2030, more directly to the following Sustainable Development Goals (SDGs):

- SDG 3, Good Health and well-being: Ensure healthy lives and promote well-being for all at all ages.
- SDG 7, Affordable and clean energy: Ensure access to affordable, reliable, sustainable and modern energy for all.
- SDG 9, Industry, Innovation and Infrastructure: Build resilient infrastructure, promote inclusive and sustainable industrialization and foster innovation.

Having also contributions to the:

- SDG 5, Gender equality: Achieve gender equality and empower all women and girls.
- SDG 10, Reduced Inequalities: Reduce inequality within and among countries.
- SDG 13, Climate action: Take urgent action to combat climate change and its impacts
- SDG 17, Partnerships for the Goals: Strengthen the means of implementation and revitalize the global partnership for sustainable development

The Project continues to contribute to UNDAF Outcome 8 / CPD Outcome 68 - All people benefit from democratic and transparent governance institutions and systems that guarantee peace consolidation, human rights and equitable service delivery. The Project will also reinforce UNDP CO efforts towards the achievement of the UNDP Strategic Plan 2018-2021, output 1.2.1 strengthening capacities at national and sub-national levels to promote inclusive local economic development and deliver basic services. The Project is in line with the UNDP Country Programme 2017-2020 and will be consider for the next CPD currently under planning.

UNDP aligns as well with the Government national priorities set out in the Government's Five Years Plan (PQG) and, at sector level, with the Health Sector Strategic Plan 2014-2019, the National HIV Strategy 2021-2025 (PEN V), contributing specifically to the Strategic Plan for Pharmaceutical Logistics (PELF) 2014-2024 and National Emergency Plan for preparedness and response to pandemic COVID-19.

The Project will support the Government's efforts towards the achievement of the Health Sector Strategic Plan targets. The stated mission of Mozambique´s Health Sector Strategic Plan is: *To lead the provision and production of better health services, universally accessible through a decentralized system that favors partnerships.*

The Project is in line with the Health Sector Strategic Plan and contributes to most of the eight strategic directions for the development of the health sector which are defined as follows:

1. Increase access and use of health care services
2. Improve the quality of services provided
3. Reduce geographic disparities between social groups in terms of access and use of services
4. Increase efficiency in the provision of services and resource use
5. Strengthen sector partnerships on the basis of mutual respect

6. Increase transparency and accountability in public resource use
7. Strengthen the Mozambican health system
8. Strengthen the Management of Public Health Emergencies



Figure 1: Key principles and pillars of the Health Sector Strategic Plan (PESS).

The Project will also directly contribute to the objective of Building Resilient and Sustainable Systems for Health that can support efficient, scale up, quality and retention in line with the Global Fund HIV Grant (MOZ-H-MOH) goals: to increase the percentage of PLHIV who know their status to 95% by 2025; to increase coverage of PLHIV on ART to 81% by 2025; to increase viral suppression among PLHIV to 73% by 2025 and to reduce the percentage of AGYW involved in risky behaviours by 10% by 2025.

The long-term objective of the Project is to strengthen the national health system by reinforcing the medical supply chain system management and strengthen decentralized laboratory capacity.

The proposed investment will build, equip and accredit drug quality control laboratories. Funding will also expand the mechanisms for supervision of the distribution chain for verified compliance with good practices on quality and safety. Waste management, interventions will entail construction of drug incinerators to serve the targeted Regions of the country.

Another key rationale for these interventions related to quality assurance is within the context of the 2017 GFATM OIG audit findings that there are limited in-country mechanisms to routinely monitor quality of medicines. Similarly, the prioritization of increased storage space responds to OIG findings as well as findings of Mozambique's 2018 Joint Assessment.

III. RESULTS AND PARTNERSHIPS

Expected Results:

UNDP will implement the Project in the framework of its Country Program Document, aligning the development support to be provided throughout this Project with other interventions aiming at improving service delivery (CPD output 4.4: Decentralization process and local governance systems enhanced to improve service delivery). The Project will contribute to strengthening capacities at national and sub-national levels to promote inclusive local economic development and deliver basic services.

As per the National Sector Legal Framework above stated, the Project is a clear opportunity to consolidate gains made in the national response and reinforce the national HIV response in Mozambique. It is aligned to the National HIV Strategy 2015-2020 (PEN IV), the current draft National HIV Strategy 2021-2025 (PEN V), Health Sector Strategic Plan (2019/2024). The adoption and scale up of "Treat All", the National Acceleration Plan and scale up of other HIV prevention interventions with commensurate community-based support, are contributing towards making the 95-95-95 goal a near reality. Despite this progress, a number of factors related to the health products management system and the current COVID-19 pandemic are militating against effective HIV response. Hence, health products management systems have emerged as a key area of investment to support the treatment scale-up envisioned in the HIV Grant and indeed TB, Malaria and the response to COVID-19 pandemic.

The Theory of Change underlying the intervention fully acknowledges the contribution that the Project will provide towards the overall objective of strengthening the national health system and is framed as follows:

Component 1: Support to PELF implementation and Waste management

It is important to increase storage and distribution capacities for drugs and consumables accompanying the treatment expansion; current storage space and conditions require further investment to strengthen quality assurance mechanisms, including the program to combat counterfeit and low-quality medicines.

Attention will be given to:

- Output 1: Improved safety, security and storage conditions of medicines, vaccines and other health products at sub-national level – through:
    - Rehabilitation of the Administrative building, including Demolition and Construction with Expansion up to more than 5200 pallet capacity for Medical Warehouse in Beira city, Sofala province, including Environmental Licensing.
    - Procurement, Installation and Training of Beira Medical Warehouse equipment: Storage Structure and Binning, Handling equipment

- Output 2: Improved systems for adequate waste management – through:
    - Procurement and installation of two (2) mobile and containerized incinerator(s) in country Regions North (Nampula) and South (Gaza), capable of running on renewable and sustainable fuel that mid-long term will reduce significantly the maintenance and management costs of the respective Medical Warehouses.

Component 2: COVID-19 emergency response

Since the beginning of the COVID-19 crisis, UNDP has aligned with the efforts of the Government of Mozambique to protect the lives and the livelihoods of its citizens, joining in on the wider United Nations response plan. Within the framework of the current Project, through the partnership with the Global Fund and the Government, National Health Institute (INS), UNDP plans to continue strengthening laboratory capacity for decentralized testing, ensuring adequate and equipped health infrastructure, including through rehabilitation and installation of prefabricated modules.
Attention will be given to:

- Output 3: Strengthened Laboratory capacity for decentralized COVID-19 testing – through:
  - Provision of additional civil works in the Laboratories of Nampula and Zambezia to address biosafety requirements as requested by INS
  - Establishment of three (3) new Laboratories in prefabricated infrastructure in Gaza, Niassa and Manica
  - Installation of Generators in the Laboratories of Beira, Tete, Inhambane, Gaza, Niassa and Manica
  - Equipment/Furniture of the Laboratories of Tete, Inhambane, Gaza, Niassa, Manica Nampula and Zambezia
  - Site evaluation and development of the Executive Projects for the renovations of the three (3) Regional Observatory Centres of INS – Maputo, Nampula and Beira

Partnerships:

Similar to the health infrastructure Projects under implementation to strengthening the National Health System, this Project is based on a strong partnership with the Ministry of Health and subnational authorities at targeted health facilities´ locations.

The MoH's timely input in all stages of the processes has proven crucial during the Project cycle implementation and their support essential in areas such as construction permits, working conditions for the labor force, and putting into action social and environmental considerations.

The Project will continue to establish linkages and synergies with other health partners and UN Agencies working on the sector, reinforcing access to health services, and facilitating decentralization. These partners will be also represented in or continued to be invited to the meetings of the Project Board. Partnerships within the framework of the National COVID-19 Emergency Response Plan will continue and new ones built as relevant to foster future cooperation and donor programmatic and financial coordination.

The Project is built on the basis of a strong relationship with the Ministry of Health and assumes an excellent coordination with the Government for the achievement of the Project´s objectives. It is essential that interventions by the MoH such as construction permits are undertaken in a harmonized manner, and that there is continuing knowledge transfer between the MoH and the PMU/UNDP during implementation of the Project. Furthermore, the Project design assumes that:

- Stakeholders (Ministry of Health, Ministry of Public Works, Hospital Administrators and local authorities) understand and subscribe to the objectives and long terms goals of the Project, and contribute to the achievement of the desired results;
- Government will be able to retain all the capacity created with the support of the Project;
- Technical expertise with the knowledge and experience required to achieve the desired results is available and can ensure innovative approaches and highly relevant contributions to the process;

- Effective coordination between UNDP programmes, with other UN agencies, and development partners ensures a collaborative approach, promotes synergies and avoids duplication of effort.

South-South and Triangular Cooperation (SSC/TrC):

The Project foresees the transfer of knowledge and skills to key personnel of the MoH at the national, provincial and district levels.

Technical and advisory support through UNDP Health Implementation Support teams will transfer knowledge from health (re)construction activities implemented at UNDP internationally, including the Sub-Saharan region. Best practices of other countries in supporting sustainable healthcare infrastructure and health system strengthening will be applied through sharing the knowledge with Project partners and stakeholders.

Such an approach ensures the growth of human capital as well as of institutional expertise, both at MoH and at UNDP Mozambique.

National experts will be contracted for the Project, ensuring use of local expertise teaming with international specialists.

During the implementation phase, further opportunities will be investigated and defined, so that South-South partnerships for capacity development can be strengthened.

Knowledge

The content of the Project provides an effective base for knowledge sharing and expansion of good practices:

- The Executive Projects can be used in future (re)constructions of health facilities.
- Technical Knowledge will be shared through involvement in the Project implementation cycle, joint monitoring visits in the field and regular progress debriefs.
- At the end of the Project, the Project Board will decide on ways to disseminate Lessons Learned and the knowledge products produced.

Sustainability and Scaling Up:

The (re)constructed and rehabilitated facilities will remain a valuable part of the Mozambican national health system, contributing to the achievement of the targets of the Health Sector Strategic Plan.

Sustaining health infrastructure requires substantial maintenance, in particular in tropical conditions, due to influence of temperature and humidity. Based on the lessons learnt from health investments in Mozambique in the past, it is now recognized that additional efforts are indispensable to sustain benefits of facilities and equipment to keep them functional for years to come.

The most essential parts of sustainability are:

a. Perception of ownership by local authorities and facility staff
b. Technical competence, continuous capacity development and adequate human resources
c. Comprehensive rolling maintenance plans
d. Dedicated maintenance financing

The Project will stand as a worthy example of organizational and technical practices for further use and scaling up. During the Project implementation UNDP will establish knowledge exchanges with governmental partners, UN Agencies, and development partners and relevant stakeholders.

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IV. PROJECT MANAGEMENT

UNDP systems and procedures are designed to guarantee transparency, accountability, cost effectiveness and value-for-money. UNDP strives to maintain high cost-efficiency, striking balance between sufficient and skilled human resources and reasonable costs, with the overall goal of optimizing the achievement of Project goals and objectives.

UNDP will build on the good practices and experience provided by the recent 'Health Systems Strengthening Project' implemented for and together with MoH.

To further guarantee efficiency and effectiveness, the following principles are implemented:

- **Strong collaboration with Government Institutions:** The Project is implemented by UNDP (according to UNDP's DIM) and in close collaboration with MoH. The MoH relevant departments for the Project will be part of the Project Board and this will facilitate greater ownership, as well as promote efficiency and flexibility in the implementation.
- **Technical expertise:** Specialized personnel will be dedicated to this Project, to allow for continuous technical follow-up towards efficient management. Advisory services will also be provided by UNDP HQ support units.
- **Joint Field Visits:** UNDP PMU staff, Government counterparts from MoH will undertake joint field visits to review Project implementation and identify and resolve implementation problems if/when they occur through created platforms in place for that end - technical working group (TWG). This TWG is mainly to analyze, evaluate and deliberate on decisions regarding the infrastructure projects and is composed by Engineers, Architects and other key technical specialists as required from the UNDP and the MoH central authorities' representatives. The decisions from this TWG are scaled up to the Project Board when impact on programmatic-financial arrangements (e.g. need for addenda to financing agreement or additional fund to accommodate technical elements identified as necessary).
- **Monitoring:** From previous experiences, UNDP will continue to implement through different levels of monitoring systems to accompany the implementation of activities and outputs so as to address the bottlenecks that arise. An external, independent final evaluation will appraise Project outputs, practices used, and lessons learned.

Project Management

Through this project, UNDP will be able to reinforce the Project Management Unit (PMU) which will provide day to day management and operational support. The PMU will be located in the UNDP CO in Maputo, though currently the UN staff remains in teleworking due to pandemic COVID-19. The PMU will benefit from the support of the UNDP common services such as procurement, financing, security, office space, communication and quality assurance. UNDP Senior Management (Deputy Resident Representative) will oversee the programme implementation and ensure robust risk management and mitigation procedures are in place.

International and local consultants will be continued to be involved in critical periods of Project life cycle towards achieving key milestones.

A joint Project Board will be established to provide strategic Project management, which includes representatives of the Ministry of Health, Ministry of Public Works, UNDP and relevant UN Agencies for the scope of works. The Project Board has specific terms of reference, as provided in annex to this Project document.

V. RESULTS FRAMEWORK

Intended Outcomes as stated in the UNDAF:

UNDAF Outcome 8/ CPD Outcome 68 - All people benefit from democratic and transparent governance institutions and systems that guarantee peace consolidation, human rights and equitable service delivery.

Outcome indicators as stated in the UNDP Country Programme Document, including baseline and targets:

UNDAF Outcome 8 /CPD Outcome 68 INDICATORS: 1. Voter turnout in parliamentary elections (BL: 48,84%, T.58%): 2. % of 2016-2020 UPR Plan of Action recommendations implemented (BL: 49% (2013), T: 100%); 3. % of State Budget allocated towards poverty reduction interventions (BL: 55% (2015), T: 60%); 4. Open budget index value (BL: 38 (2015), T. 45); 5. Overall ranking in the Mo Ibrahim African Governance index (BL: 52.3 (2015), T.54).

Applicable Output(s) from the UNDP Strategic Plan:

Output 1.2.1.: Capacities at national and sub-national levels strengthened to promote inclusive local economic development and deliver basic services including HIV and related services

Project title and Atlas Project Number: Health System Strengthening; ATLAS Project Number: 00135528

EXPECTED OUTPUTS	OUTPUT INDICATORS	DATA SOURCE	BASELINE		TARGETS (by frequency of data collection)						DATA COLLECTION METHODS & RISKS
			Value	Year	Year 1	Year 2	Year 3	Year 4	Year 5	FINAL	
Output 1: Improved safety, security and storage conditions of medicines, vaccines and other health products at sub-national level	1.1. Number of Medical Warehouses rehabilitated and Licensed	MoH	0	2021	0	1	N/A	N/A	N/A	1	Project Reports
	1.2. Number of Medical Warehouses equipped	MoH	1	2021	0	1	N/A	N/A	N/A	1	Project Reports

Output 2: Improved systems for adequate waste management	2.1. Number of Incinerators procured and installed in targeted Medical Warehouses	MoH	1	2021	0	2	N/A	N/A	N/A	2	Project Reports
Output 3: Strengthened Laboratory capacity for decentralized COVID-19 testing	3.1. Number of Laboratories rehabilitated	MoH	2	2021	2	0	N/A	N/A	N/A	2	Project Reports
	3.2. Number of Laboratories established with prefabricated infrastructure	MoH	1	2021	3	0	N/A	N/A	N/A	3	Project Reports
	3.3. Number of Laboratories equipped	MoH	0	2021	6	0	N/A	N/A	N/A	6	Project Reports
	3.4. Number of sites evaluated, and Executive Projects developed for future rehabilitation of Regional Observatory Centres (INS)	MoH	0	2021	3	0	N/A	N/A	N/A	3	Project Reports

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VI. MONITORING AND EVALUATION

In accordance with UNDP's programming policies and procedures, the Project will be monitored through the following monitoring and evaluation plans:

Monitoring Plan

Monitoring Activity	Purpose	Frequency	Expected Action	Partners (if joint)	Cost (if any)
Debrief to MoH Senior Management	Periodic update through a direct platform of reporting from UNDP to MoH Senior management, towards improvement of clear communication of Project progress.	Monthly	One (1) hour zoom meeting where UNDP presents to MoH Senior Management the progress of the month highlighting challenges and opportunities.	MoH DPC and PR; CMAM; INS and DIEH	N/A
Financial Reports	Periodic update to Principal Recipient (PR) the MoH.	Quarterly	Project delivery is presented to the PR to enable sharing of information with Department of Administration and Finance (DAF) and the Global Fund for periodic reporting towards transparency and support of MoH donor coordination.	N/A	N/A
Track results progress	Progress data against the results indicators in the RRF will be collected and analysed by the PMU to assess the progress of the Project in achieving the agreed outputs.	Bi-annually and ad-hoc as needed to be aligned with MoH needs	Slower than expected progress will be addressed by Project management.	Project Board members.	N/A
Monitor and Manage Risk	Identify specific risks that may threaten achievement of intended results. Identify and monitor risk management actions using a risk log. This includes monitoring measures and plans that may have been required as per UNDP's Social and Environmental Standards. Audits will be conducted in accordance with UNDP's audit policy to manage financial risk.	Quarterly	Risks are identified by Project management and actions are taken to manage risk. The risk log is actively maintained to keep track of identified risks and actions taken.	Project Board members.	N/A

Learn	Knowledge, good practices and lessons will be captured regularly, as well as actively sourced from other Projects and partners and integrated back into the Project.	At least annually	Relevant lessons are captured by the Project team and used to inform management decisions.	Project Board members.	N/A
Bi-Annual Project Quality Assurance	The quality of the Project will be assessed against UNDP's quality standards to identify Project strengths and weaknesses and to inform management decision making to improve the Project.	Bi-Annually	Areas of strength and weakness will be reviewed by Project management and used to inform decisions to improve Project performance.	Project Board members.	N/A
Review and Make Course Corrections	Internal review of data and evidence from all monitoring actions to inform decision making.	Bi-Annually	Performance data, risks, lessons and quality will be discussed by the Project board and used to make course corrections.	Project Board members.	N/A
Project Report	A progress report will be presented to the Project Board and key stakeholders, consisting of progress data showing the results achieved against pre-defined annual targets at the output level, the annual Project quality rating summary, an updated risk long with mitigation measures, and any evaluation or review reports prepared over the period.	Annually, and at the end of the Project (final report)	Final presentation of Project implementation cycle.	MoH PR; CMAM; INS and DIEH	N/A
Project Review (Project Board)	The Project's governance mechanism (i.e., Project board) will hold regular Project reviews to assess the performance of the Project and review the Multi-Year Work Plan to ensure realistic budgeting over the life of the Project. In the Project's final year, the Project Board shall hold an end-of Project review to capture lessons learned and discuss opportunities for scaling up and to socialize Project results and lessons learned with relevant audiences.	Semestral and *ad-hoc* as required due to emergency context progress and impact on Project operations	Any quality concerns or slower than expected progress should be discussed by the Project board and management actions agreed to address the issues identified.	Project Board members.	N/A

Evaluation Plan

Evaluation Title	Partners (if joint)	Related Strategic Plan Output	UNDAF/CPD Outcome	Planned Completion Date	Key Evaluation Stakeholders	Cost and Source of Funding
Final Evaluation	N/A	Output 1.2.1: Capacities at national and sub-national levels strengthened to promote inclusive local economic development and deliver basic services including HIV and related services	UNDAF Outcome 8/ CPD Outcome 68 - All people benefit from democratic and transparent governance institutions and systems that guarantee peace consolidation, human rights and equitable service delivery.	2022	• MoH/CMAM • MoH/INS • MoH/DIEH • MoH/Environment • MoH/Projects Department • MoH/PAV • UN Agencies • CSOs / Communities	$ 40,000

VII. MULTI-YEAR WORK PLAN

All anticipated programmatic and operational costs to support the Project, including development effectiveness and implementation support arrangements, need to be identified, estimated and fully costed in the Project budget under the relevant output(s). This includes activities that directly support the Project, such as communication, human resources, procurement, finance, audit, policy advisory, quality assurance, reporting, management, etc. All services which are directly related to the Project need to be disclosed transparently in the Project document.

EXPECTED OUTPUTS	PLANNED ACTIVITIES	Planned Budget by Year				RESPONSIBLE PARTY	PLANNED BUDGET		
		Y1	Y2	Y3	Y4		Funding Source	Budget Description	Amount (USD)
Output 1: Improved safety, security and storage conditions of medicines, vaccines and other health products at sub-national level *Gender marker: GEN1*	1.1. Number of Medical Warehouses rehabilitated and Licensed	$ 4,263,108.72	0	N/A	N/A	UNDP	GFATM	Infrastructure	$ 4,263,108.72
	1.2. Number of Medical Warehouses equipped	$ 531,558.80	0	N/A	N/A	UNDP	GFATM	Equipment	$ 531,558.80
	Sub-Total for Output 1:								**$ 4,794,667.52**
Output 2: Improved systems for adequate waste management *Gender marker: GEN1*	2.1. Number of Incinerators procured and installed in targeted Medical Warehouses	$971,481.02	0	N/A	N/A	UNDP	GFATM	Equipment	$971,481.02
	Sub-Total for Output 2:								**$971,481**
Output 3: Strengthened Laboratory capacity for decentralized COVID-19 testing *Gender marker: GEN1*	3.1. Number of Laboratories rehabilitated	$ 74,122.18	0	N/A	N/A	UNDP	GFATM	Infrastructure	$ 74,122.18
	3.2. Number of Laboratories established in prefabricated infrastructure	$ 926,750	0	N/A	N/A	UNDP	GFATM	Infrastructure	$ 926,750
	3.3. Number of Laboratories equipped	$ 611,781.42	0	N/A	N/A	UNDP	GFATM	Equipment / Furniture	$ 611,781.42

	3.4. Number of sites evaluated, and Executive Projects developed for future rehabilitation of Regional Observatory Centres (INS)	$8,623.83	0	N/A	N/A	UNDP	GFATM	Infrastructure	$8,623.83
	Sub-Total for Output 3:								**$ 1,621,277.43**
Joint Project Costs, including Final Evaluation		$ 724,963.35	$ 718,591.21						$ 1,443,554.56
General Management Support: 7%		$ 567,867.22	$ 50,301.38	N/A	N/A	UNDP	GFATM		$ 618,168.6
TOTAL									**$ 9,449,149**

Activity	2021 (CMAM & INS until 31 Dec)	2022 (only CMAM until 31 Dec)	Total	% Weight
CMAM - Infrastructure	**$ 5,766,148.03**	**$0**	**$ 5,766,148.03**	**61.0%**
Medical Warehouse Beira	**$ 4,794,667.01**	**$0**	**$4,794,667.01**	**50.7%**
Construction works	$ 3,962,132.60	$0	$ 3,962,132.60	
Supervision	$ 288,155.10	$0	$ 288,155.10	
Environmental License	$ 12,820.51	$0	$ 12,820.51	
Equipment	$ 531,558.80	$0	$ 531,558.80	
Incinerator (North - Nampula)	**$ 485,740.51**	**$0**	**$ 485,740.51**	**5.1%**
Construction works	$ 186,000.00	$0	$ 186,000.00	
Supervision	$ 18,600.00	$0	$ 18,600.00	
Equipment	$ 268,320.00	$0	$ 268,320.00	
Environmental License	$ 12,820.51	$0	$ 12,820.51	
Incinerator (South - Gaza)	**$ 485,740.51**	**$0**	**$ 485,740.51**	

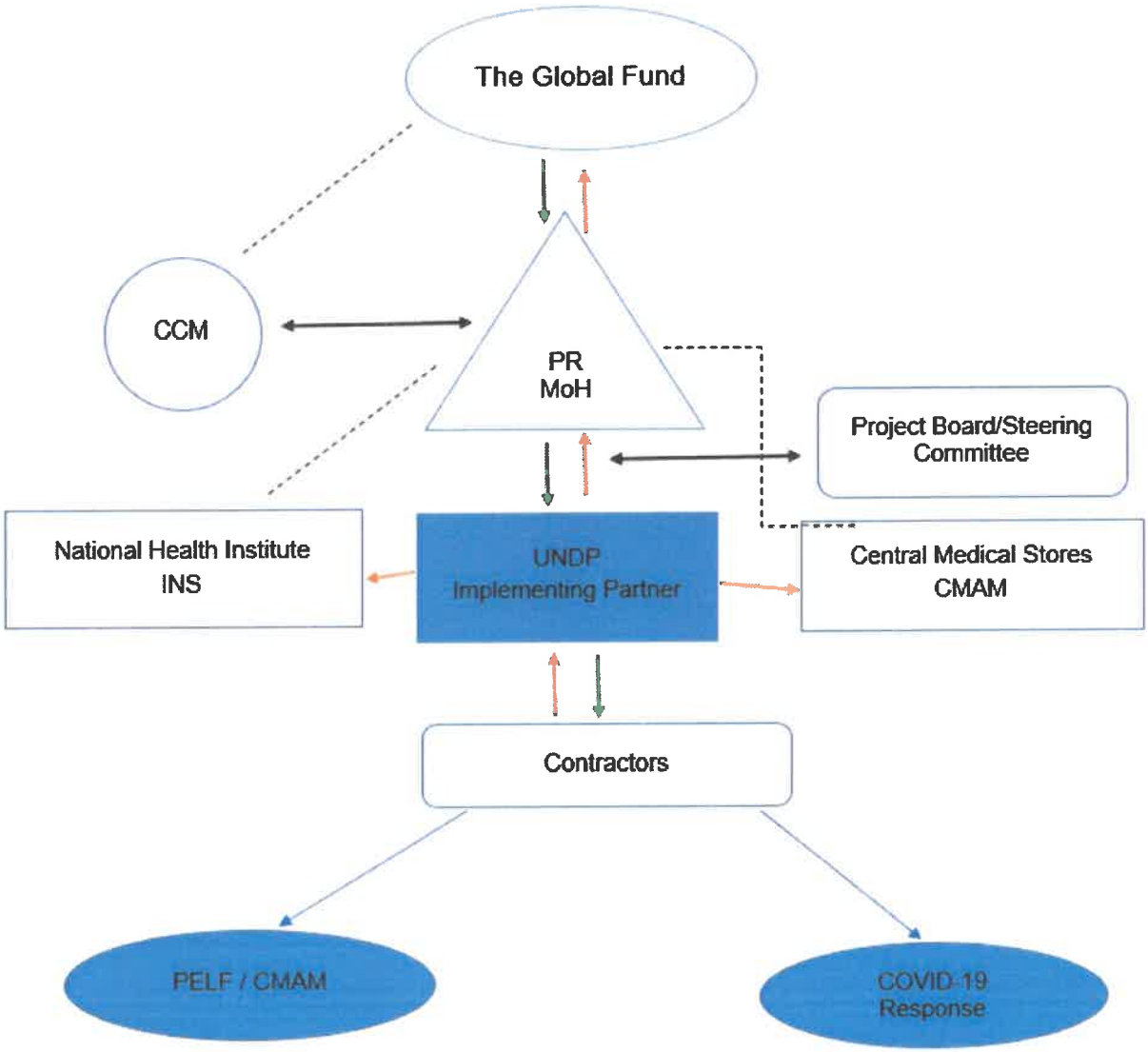
Construction works	$ 186,000.00	$0	$ 186,000.00	
Supervision	$ 18,600.00	$0	$ 18,600.00	
Environmental License	$ 12,820.51	$0	$ 12,820.51	
Equipment	$ 268,320.00	$0	$ 12,820.51	
COVID-19 INS: Infrastructure	**$ 1,612,653.59**	**$0**	**$ 1,612,653.59**	**17.1%**
Establishment of 3 new and equipped Laboratories for COVID-19 testing	**$ 1,237,655.10**	**$0**	**$ 1,237,655.10**	**13.1%**
Construction works	$ 834,750.00	$0	$ 834,750.00	
Supervision	$ 92,000.00	$0	$ 92,000.00	
Laboratory equipment	$ 95,251.38	$0	$ 95,251.38	
Generators	$ 215,653.73	$0	$ 215,653.73	
Acquisition and installation of Generators and Lab equipment in Beira, Tete and Inhambane	**$ 300,876.31**	**$0**	**$ 300,876.31**	**3.2%**
Construction works	$0	$0	$0	
Supervision	$ 18,000.00	$0	$ 18,000.00	
Equipment	$ 215,653.73	$0	$ 215,653.73	
Furniture	$ 67,222.58	$0	$ 67,222.58	
Amendment to Current Labs	**$ 74,122.18**	**$0**	**$ 74,122.18**	**0.8%**
Administrative Costs	**$ 733,587.18**	**$ 718,591.21**	**$ 1,452,178.39**	**15.4%**
Human Resources	**$ 592,705.42**	**$671,817.60**	**$ 1,264,523.01**	**13.4%**
UNDP staff in place	$ 220,314.74	$ 246,467.65	$ 466,782.38	
Volunteers	$ 83,245.68	$ 91,164.15	$ 174,409.83	
Specific expertise / Engineering validation	$ 56,000.00	$ 14,000.00	$ 70,000.00	
Expert Positions	$ 233,145.00	$ 320,185.80	$ 553,330.80	
Administrative activities CMAM	**$51,003.75**	**$35,294.34**	**$ 86,298.09**	**0.9%**
Field missions	$ 18,854.93	$ 2,876.74	$ 21,731.67	
Fuel for UNDP vehicle	$0	$0	$0	
Furnitures for PMU staff	$ 6,802.00	$0	$ 6,802.00	
Maintenance of UNDP vehicle	$0	$0	$0	

Office supplies	$ 1,260.00	$ 1,680.00	$ 2,940.00	
IT Equipment	$0	$0	$0	
Office rent	$ 4,500.00	$ 6,000.00	$ 10,500.00	
Communication Services	$ 3,553.20	$ 4,737.60	$ 8,290.80	
Advertisement newspaper	$ 8,275.00	$0	$ 8,275.00	
VAT booklet	$ 206.90	$0	$ 206.90	
Translation Services	$ 7,551.72	$0	$ 7,551.72	
Final evaluation	$0	$ 20,000.00	$ 20,000.00	
Additional costs Incinerator (South – Gaza)	**$23,038.23**	**$11,479.27**	**$34,517.50**	
Monitoring missions of UNDP staff to Gaza (Incinerator)	$ 5,739.64	$ 5,739.64	$ 11,479.27	
Supervision missions of UNDP Management to Gaza (1 Programme Manager & 3 Senior Engineer)	$ 8,609.45	$ 5,739.64	$ 14,349.09	
Advertisement	$ 4,965.00	$0	$ 4,965.00	
Translation Services	$ 3,724.14	$0	$ 3,724.14	
Administrative activities COVID-19_INS	**$66,839.79**	**$0**	**$ 66,839.79**	**0.7%**
Preparation phase / sites assessments	$ 8,623.83	$0	$ 8,623.83	
Field missions	$ 23,237.62	$0	$ 23,237.62	
Office supplies	$ 1,260.00	$0	$ 1,260.00	
Communication Services	$ 2,960.07	$0	$ 2,960.07	
Printing and publishing	$ 3,310.00	$0	$ 3,310.00	
Translation Services	$ 7,448.28	$0	$ 7,448.28	
Final evaluation	$ 20,000.00	$0	$ 20,000.00	
Sub total	**$ 8,112,388.80**	**$ 718,591.21**	**$ 8,830,980.01**	
GMS (7%)	$ 567,867.22	$ 50,301.38	$ 618,168.60	6.5%
Total for NFM3_CMAM & INS	**$ 8,680,256.02**	**$ 768,892.59**	**$ 9,449,148.61**	100.0%

Figure 2: Summary of approved joint budget by MoH-UNDP-GFATM, 2021-2022.

VIII. GOVERNANCE AND MANAGEMENT ARRANGEMENTS

UNDP will execute the Project in line with the UNDP's Direct Implementation Modality (DIM) procedures and guidelines. UNDP currently works as the nominated Implementing Partner under the general guidance of the Country Coordination Mechanism (CCM) and Principal Recipient the MoH, and is responsible for programme management, financial accountability, procurement of goods & services and Monitoring and Evaluation.



Legenda:

	Reporting
	Funding
	Deliverable
	Coordination

Figure 3: Project implementation mapping.

UNDP has strong systems which ensure requisite resources are available for identifying and assessing contractors and for overseeing the contractors once engaged. Additionally, UNDP carefully addresses and manages potential risks involved in working with responsible partners (contractors). This includes critical aspects such as project implementation and oversight arrangements, importation and tax exemption agreements, fund transfer and banking arrangements, privileges and immunities, protocols for audit and investigations.

UNDP will leverage this significant experience in understanding the complexities and challenges of managing civil works interventions, utilizing its financial, M&E, procurement and management systems, rules, regulations, policies and processes to ensure successful implementation of the program and achievement of the project objectives.

UNDP has a strong track record of implementing civil works and supply chain activities (with strong risk management and sustainability components) and importantly has a well-established presence in Mozambique.

Project implementation will comply with the UNDP Programme and Operations Policies and Procedures (POPP). UNDP has a dedicated Global Fund Partnership Team (HIV, Health and Development, Bureau of Policy and Programme Support) (including procurement and capacity development advisory team) in place. Based in New York, Copenhagen, Geneva and Addis Ababa, the Team's primary goal is to provide quality and timely support to Country Offices to implement high performing Global Fund grants and to manage UNDP's partnership with the Global Fund at the corporate level.

The institutional arrangement under the previous cycle will continue during the life of the new grant.

In the past 2 years, UNDP also established a sub-office in Beira city and is currently establishing a new sub-office in Pemba. This decentralized re-structure will support the field operations in Center and North Regions.

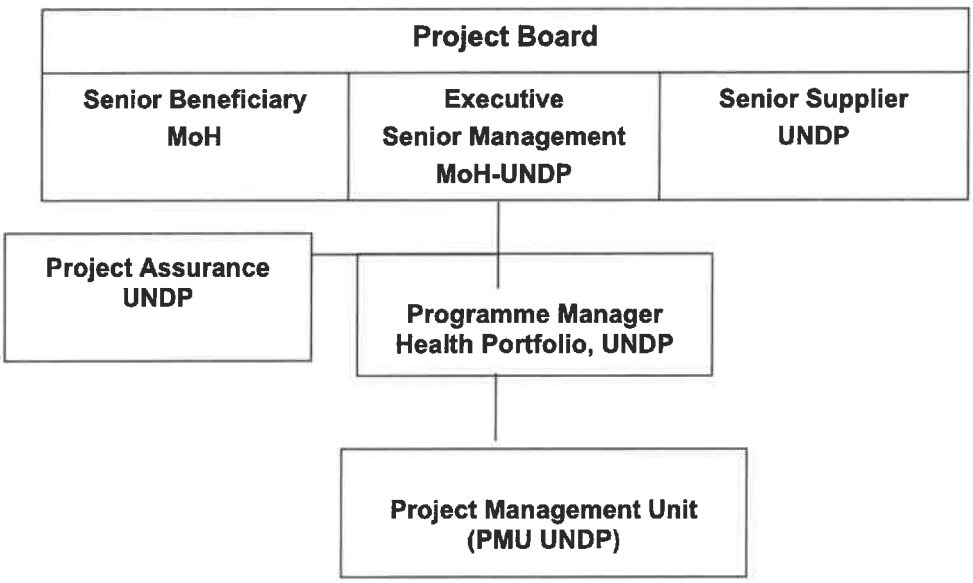


Figure 4: Project Organizational Structure / Governance mechanism.

The new Project cycle comprehends core positions for Senior Engineering and Operations (Procurement and Finance) Management, to increase the capacity of response to the different demands in current complex context of implementation not only due to pandemic COVID-19 but also due to the open conflict in the North.

The structure of the 2021-2022 UNDP PMU is as follows:

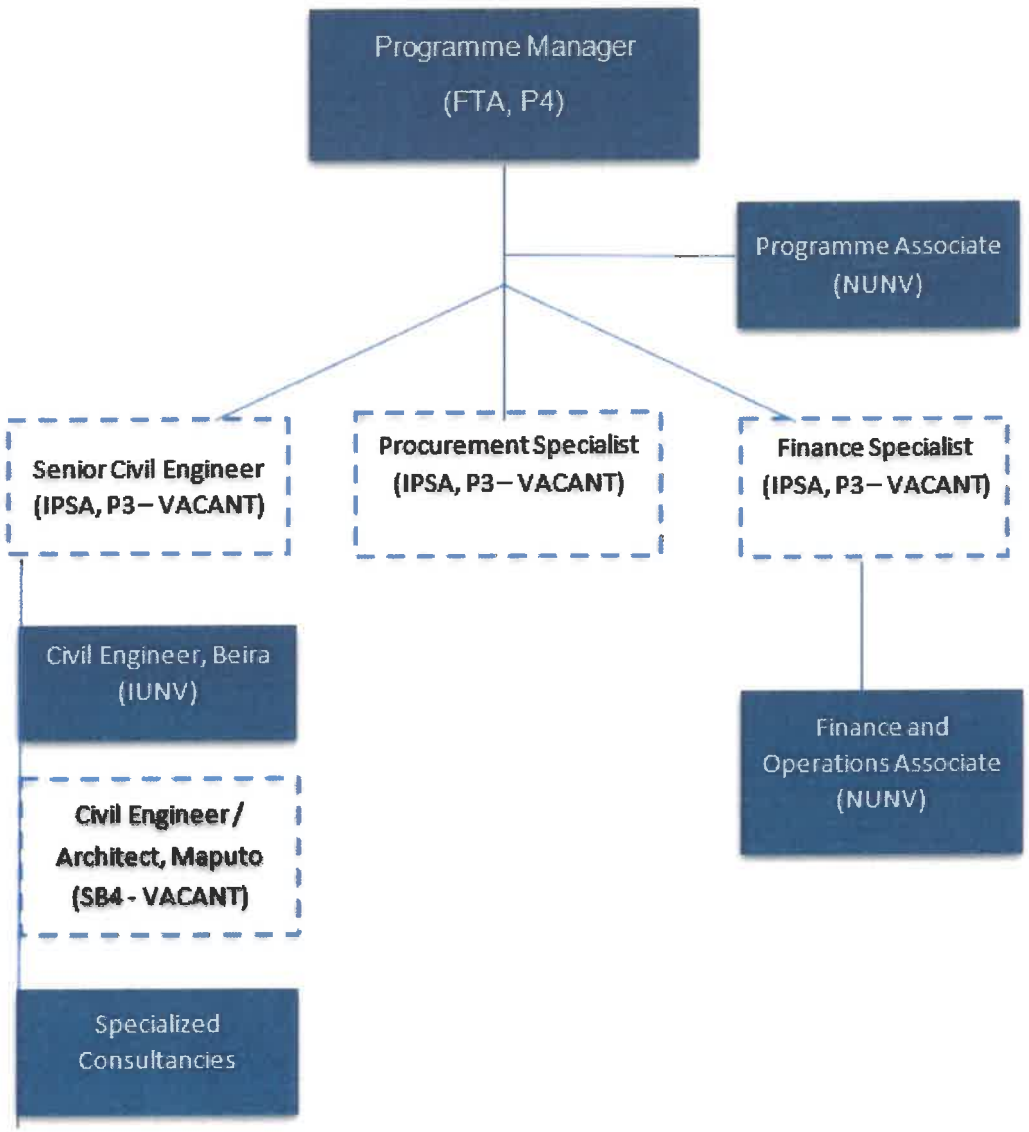


Figure 5: UNDP PMU 2021-2022 approved organogram, reflecting the positions funded by The Global Fund.

IX. LEGAL CONTEXT

This Project will be implemented by UNDP ("Implementing Partner") in accordance with its financial regulations, rules, practices and procedures only to the extent that they do not contravene the principles of the Financial Regulations and Rules of UNDP. Where the financial governance of an Implementing Partner does not provide the required guidance to ensure best value for money, fairness, integrity, transparency, and effective international competition, the financial governance of UNDP shall apply.

X. RISK MANAGEMENT

UNDP (DIM)

1. UNDP as the Implementing Partner will comply with the policies, procedures and practices of the United Nations Security Management System (UNSMS.)

2. UNDP as the Implementing Partner will undertake all reasonable efforts to ensure that none of the [Project funds][1] [UNDP funds received pursuant to the Project Document][2] are used to provide support to individuals or entities associated with terrorism and that the recipients of any amounts provided by UNDP hereunder do not appear on the list maintained by the Security Council Committee established pursuant to resolution 1267 (1999). The list can be accessed via http://www.un.org/sc/committees/1267/aq_sanctions_list.shtml. This provision must be included in all sub-contracts or sub-agreements entered into under this Project Document.

3. Social and environmental sustainability will be enhanced through application of the UNDP Social and Environmental Standards (http://www.undp.org/ses) and related Accountability Mechanism (http://www.undp.org/secu-srm).

4. UNDP as the Implementing Partner will: (a) conduct Project and programme-related activities in a manner consistent with the UNDP Social and Environmental Standards, (b) implement any management or mitigation plan prepared for the Project or programme to comply with such standards, and (c) engage in a constructive and timely manner to address any concerns and complaints raised through the Accountability Mechanism. UNDP will seek to ensure that communities and other Project stakeholders are informed of and have access to the Accountability Mechanism.

5. In the implementation of the activities under this Project Document, UNDP as the Implementing Partner will handle any sexual exploitation and abuse ("SEA") and sexual harassment ("SH") allegations in accordance with its regulations, rules, policies and procedures.

6. All signatories to the Project Document shall cooperate in good faith with any exercise to evaluate any programme or Project-related commitments or compliance with the UNDP Social and Environmental Standards. This includes providing access to Project sites, relevant personnel, information, and documentation.

7. UNDP as the Implementing Partner will ensure that the following obligations are binding on each responsible party, subcontractor and sub-recipient:

    a. Consistent with the Article III of the SBAA *[or the Supplemental Provisions to the Project Document]*, the responsibility for the safety and security of each responsible party, subcontractor and sub-recipient and its personnel and property, and of UNDP's property in such responsible party's, subcontractor's and sub-recipient's custody, rests with such responsible party, subcontractor and sub-recipient. To this end, each responsible party, subcontractor and sub-recipient shall:

[1] To be used where UNDP is the Implementing Partner

[2] To be used where the UN, a UN fund/programme or a specialized agency is the Implementing Partner

i. put in place an appropriate security plan and maintain the security plan, taking into account the security situation in the country where the Project is being carried;

ii. assume all risks and liabilities related to such responsible party's, subcontractor's and sub-recipient's security, and the full implementation of the security plan.

b. UNDP reserves the right to verify whether such a plan is in place, and to suggest modifications to the plan when necessary. Failure to maintain and implement an appropriate security plan as required hereunder shall be deemed a breach of the responsible party's, subcontractor's and sub-recipient's obligations under this Project Document.

c. In the performance of the activities under this Project, UNDP as the Implementing Partner shall ensure, with respect to the activities of any of its responsible parties, sub-recipients and other entities engaged under the Project, either as contractors or subcontractors, their personnel and any individuals performing services for them, that those entities have in place adequate and proper procedures, processes and policies to prevent and/or address SEA and SH.

d. Each responsible party, subcontractor and sub-recipient will take appropriate steps to prevent misuse of funds, fraud or corruption, by its officials, consultants, subcontractors and sub-recipients in implementing the Project or programme or using the UNDP funds. It will ensure that its financial management, anti-corruption and anti-fraud policies are in place and enforced for all funding received from or through UNDP.

e. The requirements of the following documents, then in force at the time of signature of the Project Document, apply to each responsible party, subcontractor and sub-recipient: (a) UNDP Policy on Fraud and other Corrupt Practices and (b) UNDP Office of Audit and Investigations Investigation Guidelines. Each responsible party, subcontractor and sub-recipient agrees to the requirements of the above documents, which are an integral part of this Project Document and are available online at www.undp.org.

f. In the event that an investigation is required, UNDP will conduct investigations relating to any aspect of UNDP programmes and Projects. Each responsible party, subcontractor and sub-recipient will provide its full cooperation, including making available personnel, relevant documentation, and granting access to its (and its consultants', subcontractors' and sub-recipients') premises, for such purposes at reasonable times and on reasonable conditions as may be required for the purpose of an investigation. Should there be a limitation in meeting this obligation, UNDP shall consult with it to find a solution.

g. Each responsible party, subcontractor and sub-recipient will promptly inform UNDP as the Implementing Partner in case of any incidence of inappropriate use of funds, or credible allegation of fraud or corruption with due confidentiality.

Where it becomes aware that a UNDP Project or activity, in whole or in part, is the focus of investigation for alleged fraud/corruption, each responsible party, subcontractor and sub-recipient will inform the UNDP Resident Representative/Head of Office, who will promptly inform UNDP's Office of Audit and Investigations (OAI). It will provide regular updates to the head of UNDP in the country and OAI of the status of, and actions relating to, such investigation.

h. UNDP will be entitled to a refund from the responsible party, subcontractor or sub-recipient of any funds provided that have been used inappropriately, including through fraud or corruption, or otherwise paid other than in accordance with the terms and conditions of the Project Document. Such amount may be deducted by UNDP from any payment due to the responsible party, subcontractor or sub-recipient under this or any other agreement.

Where such funds have not been refunded to UNDP, the responsible party, subcontractor or sub-recipient agrees that donors to UNDP (including the Government) whose funding is the source, in whole or in part, of the funds for the activities under this Project Document, may seek recourse to such responsible party, subcontractor or sub-recipient for the recovery of any funds determined by UNDP to have been used inappropriately, including through fraud or corruption, or otherwise paid other than in accordance with the terms and conditions of the Project Document.

Note: The term "Project Document" as used in this clause shall be deemed to include any relevant subsidiary agreement further to the Project Document, including those with responsible parties, subcontractors and sub-recipients.

i. Each contract issued by the responsible party, subcontractor or sub-recipient in connection with this Project Document shall include a provision representing that no fees, gratuities, rebates, gifts, commissions or other payments, other than those shown in the proposal, have been given, received, or promised in connection with the selection process or in contract execution, and that the recipient of funds from it shall cooperate with any and all investigations and post-payment audits.

j. Should UNDP refer to the relevant national authorities for appropriate legal action any alleged wrongdoing relating to the Project or programme, the Government will ensure that the relevant national authorities shall actively investigate the same and take appropriate legal action against all individuals found to have participated in the wrongdoing, recover and return any recovered funds to UNDP.

k. Each responsible party, subcontractor and sub-recipient shall ensure that all of its obligations set forth under this section entitled "Risk Management" are passed on to its subcontractors and sub-recipients and that all the clauses under this section entitled "Risk Management Standard Clauses" are adequately reflected, *mutatis mutandis*, in all its sub-contracts or sub-agreements entered into further to this Project Document.

XI. ANNEXES

1. **Project Quality Assurance (PQA) Report**
2. **Social and Environmental Screening (SES)**
3. **Risk Analysis**
4. **Project Board Terms of Reference**

PROJECT QA ASSESSMENT: DESIGN AND APPRAISAL

OVERALL PROJECT

EXEMPLARY (5) ●●●●●	HIGHLY SATISFACTORY (4) ●●●●○	SATISFACTORY (3) ●●●○○	NEEDS IMPROVEMENT (2) ●●○○○	INADEQUATE (1) ●○○○○
At least four criteria are rated Exemplary, and all criteria are rated High or Exemplary.	All criteria are rated Satisfactory or higher, and at least four criteria are rated High or Exemplary.	At least six criteria are rated Satisfactory or higher, and only one may be rated Needs Improvement. The Principled criterion must be rated Satisfactory or above.	At least three criteria are rated Satisfactory or higher, and only four criteria may be rated Needs Improvement.	One or more criteria are rated Inadequate, or five or more criteria are rated Needs Improvement.

DECISION

- **APPROVE** – the project is of sufficient quality to be approved in its current form. Any management actions must be addressed in a timely manner.
- **APPROVE WITH QUALIFICATIONS** – the project has issues that must be addressed before the project document can be approved. Any management actions must be addressed in a timely manner.
- **DISAPPROVE** – the project has significant issues that should prevent the project from being approved as drafted.

RATING CRITERIA

For all questions, select the option that best reflects the project

STRATEGIC

1. Does the project specify how it will contribute to higher level change through linkage to the programme's Theory of Change?

- **3:** The project is clearly linked to the programme's theory of change. It has an explicit change pathway that explains how the project will contribute to outcome level change and why the project's strategy will likely lead to this change. This analysis is backed by credible evidence of what works effectively in this context and includes assumptions and risks.
- **2:** The project is clearly linked to the programme's theory of change. It has a change pathway that explains how the project will contribute to outcome-level change and why the project strategy will likely lead to this change.
- **1:** The project document may describe in generic terms how the project will contribute to development results, without an explicit link to the programme's theory of change.

**Note: Projects not contributing to a programme must have a project-specific Theory of Change. See alternative question under the lightbulb for these cases.*

3 | 2 | 1

Evidence

The project document describes in detail how the project will contribute to development results. Including UNDAF Outcome 8/ CPD Outcome 68 - All people benefit from democratic and transparent governance institutions and systems that guarantee peace consolidation, human rights and equitable service delivery.

An explicit link to the programme's theory of change is done, detailing the strategy that will led to outcome level change. There are also clearly identified the goals that the project is contributing for HIV components of the Global Fund Grants.

3 | 2 | 1 | 2

2. Is the project aligned with the UNDP Strategic Plan? • **3:** The project responds to at least one of the development settings as specified in the Strategic Plan[1] and adapts at least one Signature Solution[2]. The project's RRF includes all the relevant SP output indicators. *(all must be true)* • **2:** The project responds to at least one of the development settings as specified in the Strategic Plan[4]. The project's RRF includes at least one SP output indicator, if relevant. *(both must be true)* • **1:** The project responds to a partner's identified need, but this need falls outside of the UNDP Strategic Plan. Also select this option if none of the relevant SP indicators are included in the RRF.	1 The Project reinforces UNDP CO efforts towards the achievement of the UNDP Strategic Plan 2018-2021, output 1.2.1 strengthening capacities at national and sub-national levels to promote inclusive local economic development and deliver basic services. The Project is in line with the UNDP Country Programme 2017-2020 and will be consider for the next CPD currently under planning. UNDP aligns as well with the Government national priorities set out in the Government's Five Years Plan (PQG) and, at sector level, with the Health Sector Strategic Plan 2014-2019, the National HIV Strategy 2021-2025 (PEN V), contributing specifically to the Strategic Plan for Pharmaceutical Logistics (PELF) 2014-2024 and National Emergency Plan for preparedness and response to pandemic COVID-19.
3. Is the project linked to the programme outputs? (i.e., UNDAF Results Group Workplan/CPD, RPD or Strategic Plan IRRF for global projects/strategic interventions not part of a programme)	Yes / No
RELEVANT	
4. Does the project target groups left furthest behind? • **3:** The target groups are clearly specified, prioritising discriminated and marginalized groups left furthest behind, identified through a rigorous process based on evidence. • **2:** The target groups are clearly specified, prioritizing groups left furthest behind. • **1:** The target groups are not clearly specified. *Note: Management Action must be taken for a score of 1. *Projects that build institutional capacity should still identify targeted groups to justify support*	3 / 2 / 1 **Evidence** The targets groups are clearly identified in the context of HIV and COVID-19, to which the project is related to. The context analysis, with evidence of source of data, is described, and clearly refers to the geographic aspect, gender and Age, key populations and vulnerable groups.

[1] The three development settings in UNDP's 2018-2021 Strategic Plan are: a) Eradicate poverty in all its forms and dimensions; b) Accelerate structural transformations for sustainable development; and c) Build resilience to shocks and crises

[2] The six Signature Solutions of UNDP's 2018-2021 Strategic Plan are: a) Keeping people out of poverty; b) Strengthen effective, inclusive and accountable governance; c) Enhance national prevention and recovery capacities for resilient societies; d) Promote nature based solutions for a sustainable planet; e) Close the energy gap; and f) Strengthen gender equality and the empowerment of women and girls.

5. Have knowledge, good practices, and past lessons learned of UNDP and others informed the project design? • **3:** Knowledge and lessons learned backed by credible evidence from sources such as evaluation, corporate policies/strategies, and/or monitoring have been explicitly used, with appropriate referencing, to justify the approach used by the project. • **2:** The project design mentions knowledge and lessons learned backed by evidence/sources, but have not been used to justify the approach selected. • **1:** There is little or no mention of knowledge and lessons learned informing the project design. Any references made are anecdotal and not backed by evidence. *Note: Management Action or strong management justification must be given for a score of 1	3 2 1 **Evidence** UNDP is a founding cosponsor of the Joint UN Programme on HIV/AIDS (UNAIDS), a partner of the Global Fund to Fight AIDS, Tuberculosis and Malaria, and a co-sponsor of several other international health partnerships. UNDP's work on HIV, health and development leverages the organization's core strengths and mandates in human development, governance and capacity development to complement the efforts of specialist health-focused UN agencies. UNDP partners with the Global Fund to support and strengthen national responses to HIV, Tuberculosis and malaria. The objective is to ensure access to quality health services for all and enable all those living or affected by the diseases to live healthy, productive lives. The partnership leverages UNDP's mandate to strengthened institutions to deliver universal access to basic services and rebuilding resilient health services in crisis and post-crisis settings, thus making a significant and measurable contribution to Outcomes 3 and 6 of UNDP's Strategic Plan and to SDG 3. The project has been designed drawing on UNDPs experience. The design of the project reflects the corporately identified risks as well as OAI findings and recommendations in similar projects.
6. Does UNDP have a clear advantage to engage in the role envisioned by the project vis-à-vis national/regional/global partners and other actors? • **3:** An analysis has been conducted on the role of other partners in the area where the project intends to work, and credible evidence supports the proposed engagement of UNDP and partners through the project, including identification of potential funding partners. It is clear how results achieved by partners will complement the project's intended results and a communication strategy is in place to communicate results and raise visibility vis-à-vis key partners. Options for south-south and	3 2 1 **Evidence** The project is built on the basis of a strong relationship with the Ministry of Health and assumes an excellent coordination with the Government for the achievement of the project's objectives.

triangular cooperation have been considered, as appropriate. *(all must be true)* • **2:** Some analysis has been conducted on the role of other partners in the area where the project intends to work, and relatively limited evidence supports the proposed engagement of and division of labour between UNDP and partners through the project, with unclear funding and communications strategies or plans. • **1:** No clear analysis has been conducted on the role of other partners in the area that the project intends to work. There is risk that the project overlaps and/or does not coordinate with partners' interventions in this area. Options for south-south and triangular cooperation have not been considered, despite its potential relevance. *Note: Management Action or strong management justification must be given for a score of 1	Stakeholders (Ministry of Health, Ministry of Public Works, Hospital Administrators and local authorities) understand and subscribe to the objectives and long terms goals of the project, and contribute to the achievement of the desired results; Effective coordination between UNDP programmes, with other UN agencies, and development partners ensures a collaborative approach, promotes synergies and avoids duplication of effort. South-South and Triangular Cooperation (SSC/TrC): The project foresees the transfer of knowledge and skills to key personnel of the MoH at the national, provincial and local levels. UNDP Global Health and Development and Procurement teams will transfer knowledge from health (re)construction activities including the Sub-Saharan region. Best practices of other countries in supporting sustainable healthcare infrastructure and health system strengthening will be applied through sharing the knowledge with project partners and stakeholders.
PRINCIPLED	
7. Does the project apply a human rights-based approach? • **3:** The project is guided by human rights and incorporates the principles of accountability, meaningful participation, and non-discrimination in the project's strategy. The project upholds the relevant international and national laws and standards. Any potential adverse impacts on enjoyment of human rights were rigorously identified and assessed as relevant, with appropriate mitigation and management measures incorporated into project design and budget. *(all must be true)* • **2:** The project is guided by human rights by prioritizing accountability, meaningful participation and non-discrimination. Potential adverse impacts on enjoyment of human rights were identified and assessed as relevant, and appropriate mitigation and management measures incorporated into the project design and budget. *(both must be true)* • **1:** No evidence that the project is guided by human rights. Limited or no evidence that potential adverse impacts on enjoyment of human rights were considered. *Note: Management action or strong management justification must be given for a score of 1	3 1 **Evidence** Health is both a driver and a result of development, it is a human right. UNDP focuses on addressing social, economic and environmental determinants, which are primarily responsible for health inequalities. This project supports the Ministry of Health in responding to the acute crisis that is the COVID 19 global pandemic. The main activities of the project address immediate needs while contributing to a resilient and sustainable health systems. The project will work in different provinces to ensure that target and beneficiary communities have access to treatment and care, receive goods in the right quantity, in good condition, at the right time, at the right cost.

	Accountability and meaningful participation, in a non-discriminatory manner, are considered not only in the project strategy, but also in the Project Board, as it can receive beneficiaries, according to the discussion items on the meeting agenda.
8. Does the project use gender analysis in the project design? • **3:** A participatory gender analysis has been conducted and results from this gender analysis inform the development challenge, strategy and expected results sections of the project document. Outputs and indicators of the results framework include explicit references to gender equality, and specific indicators measure and monitor results to ensure women are fully benefitting from the project. *(all must be true)* • **2:** A basic gender analysis has been carried out and results from this analysis are scattered (i.e., fragmented and not consistent) across the development challenge and strategy sections of the project document. The results framework may include some gender sensitive outputs and/or activities but gender inequalities are not consistently integrated across each output. *(all must be true)* • **1:** The project design may or may not mention information and/or data on the differential impact of the project's development situation on gender relations, women and men, but the gender inequalities have not been clearly identified and reflected in the project document. *Note: Management Action or strong management justification must be given for a score of 1	3 **2** 1 Evidence Mozambique is 142th out of 164 countries on UNDP's Gender Equality Index. Strongly gendered cultural norms impose low decision-making power of women in questions affecting their every-day life and movements outside of home, including their access to health services. There may be an inequality related use of centralized facilities by female patients. Decentralized health facilities offers a broader access to treatment and care, with the possibilities for women to find adequate health services near their home.
9. Did the project support the resilience and sustainability of societies and/or ecosystems? • **3:** Credible evidence that the project addresses sustainability and resilience dimensions of development challenges, which are integrated in the project strategy and design. The project reflects the interconnections between the social, economic and environmental dimensions of sustainable development. Relevant shocks, hazards and adverse social and environmental impacts have been identified and rigorously assessed with appropriate management and mitigation measures incorporated into project design and budget. *(all must be true)*. • **2:** The project design integrates sustainability and resilience dimensions of development challenges. Relevant shocks, hazards and adverse social and environmental impacts have been identified and assessed, and relevant management and mitigation measures incorporated into project design and budget. *(both must be true)* • **1:** Sustainability and resilience dimensions and impacts were not adequately considered. *Note: Management action or strong management justification must be given for a score of 1	3 **2** 1 **Evidence** The project addresses strengthening environmental sustainability through improving the safety, security and storage conditions of medicines, vaccines and other health products. Any equipment procured will strictly follow international and UNDP environmental safe guards. Resilience and sustainability of societies and/or ecosystems was assessed through SES assessment.
10. Has the Social and Environmental Screening Procedure (SESP) been conducted to identify potential social and environmental impacts and risks? The SESP is not required for projects in which UNDP is Administrative Agent only and/or projects comprised solely of reports, coordination of events, trainings, workshops, meetings, conferences and/or communication materials and	**Yes** No SESP Not Required

information dissemination. [if yes, upload the completed checklist. If SESP is not required, provide the reason for the exemption in the evidence section.]	

MANAGEMENT & MONITORING

11. Does the project have a strong results framework? • **3:** The project's selection of outputs and activities are at an appropriate level. Outputs are accompanied by SMART, results-oriented indicators that measure the key expected development changes, each with credible data sources and populated baselines and targets, including gender sensitive, target group focused, sex-disaggregated indicators where appropriate. *(all must be true)* • **2:** The project's selection of outputs and activities are at an appropriate level. Outputs are accompanied by SMART, results-oriented indicators, but baselines, targets and data sources may not yet be fully specified. Some use of target group focused, sex-disaggregated indicators, as appropriate. *(all must be true)* • **1:** The project's selection of outputs and activities are not at an appropriate level; outputs are not accompanied by SMART, results-oriented indicators that measure the expected change and have not been populated with baselines and targets; data sources are not specified, and/or no gender sensitive, sex-disaggregation of indicators. *(if any is true)* *Note: Management Action or strong management justification must be given for a score of 1	3 2 1 **Evidence** Due to the nature of the project, the expected results and measurement of activities are part of the work plan. Although the results framework is appropriate, they are SMART and the data sources are very clear.
12. Is the project's governance mechanism clearly defined in the project document, including composition of the project board? • **3:** The project's governance mechanism is fully defined. Individuals have been specified for each position in the governance mechanism (especially all members of the project board.) Project Board members have agreed on their roles and responsibilities as specified in the terms of reference. The ToR of the project board has been attached to the project document. *(all must be true).* • **2:** The project's governance mechanism is defined; specific institutions are noted as holding key governance roles, but individuals may not have been specified yet. The project document lists the most important responsibilities of the project board, project director/manager and quality assurance roles. *(all must be true)* • **1:** The project's governance mechanism is loosely defined in the project document, only mentioning key roles that will need to be filled at a later date. No information on the responsibilities of key positions in the governance mechanism is provided. *Note: Management Action or strong management justification must be given for a score of 1	3 2 1 **Evidence** The Project Board is the central organ to ensure governance, strategic oversight, and input into decision making, as well as overall guidance. The Project Board will serve as a platform that key stakeholders can use to discuss the overall direction of the project implementation, as well as to make strategic decisions to ensure the best use of resources to achieve goals and objectives. The Project Board will be co-chaired by the UNDP Deputy Resident Representative and the Ministry of Health. Other UN Agencies and main stakeholders will be members of the Board.
13. Have the project risks been identified with clear plans stated to manage and mitigate each risk? • **3:** Project risks related to the achievement of results are fully described in the project risk log, based on comprehensive analysis drawing on the programme's theory of change, Social and Environmental Standards and screening, situation analysis, capacity assessments and other analysis such as funding potential and reputational risk. Risks have been identified through a consultative process with key internal and external stakeholders. Clear and complete plan in place to manage and mitigate	3 2 1 **Evidence** The Project is to be managed in the DIM modality, in itself a a risk reduction strategy. In addition a rigorous risk registration matrix was developed, with

each risk, reflected in project budgeting and monitoring plans. *(both must be true)* • **2:** Project risks related to the achievement of results are identified in the initial project risk log based on a minimum level of analysis and consultation, with mitigation measures identified for each risk. • **1:** Some risks may be identified in the initial project risk log, but no evidence of consultation or analysis and no clear risk mitigation measures identified. This option is also selected if risks are not clearly identified and/or no initial risk log is included with the project document. *Note: Management Action must be taken for a score of 1	a clear and concrete plan to manage and mitigate each risk.	
EFFICIENT		
14. Have specific measures for ensuring cost-efficient use of resources been explicitly mentioned as part of the project design? This can include, for example: i) using the theory of change analysis to explore different options of achieving the maximum results with the resources available; ii) using a portfolio management approach to improve cost effectiveness through synergies with other interventions; iii) through joint operations (e.g., monitoring or procurement) with other partners; iv) sharing resources or coordinating delivery with other projects, v) using innovative approaches and technologies to reduce the cost of service delivery or other types of interventions. *(Note: Evidence of at least one measure must be provided to answer yes for this question)*	YES (3) Strong collaboration with Government Institutions: The project will be implemented by UNDP (according to DIM - direct implementation modality) and in close collaboration with MoH. Technical specialists (civil engineers) will be recruited and will be part of the project management unit. In addition, the Government will be part of the Project Board and this will facilitate greater ownership of the project and ensure efficiency and flexibility in implementation.	No (1)
15. Is the budget justified and supported with valid estimates? • **3:** The project's budget is at the activity level with funding sources, and is specified for the duration of the project period in a multi-year budget. Realistic resource mobilisation plans are in place to fill unfunded components. Costs are supported with valid estimates using benchmarks from similar projects or activities. Cost implications from inflation and foreign exchange exposure have been estimated and incorporated in the budget. Adequate costs for monitoring, evaluation, communications and security have been incorporated. • **2:** The project's budget is at the activity level with funding sources, when possible, and is specified for the duration of the project in a multi-year budget, but no funding plan is in place. Costs are supported with valid estimates based on prevailing rates. • **1:** The project's budget is not specified at the activity level, and/or may not be captured in a multi-year budget.	3 2 1 **Evidence** The project budget was developed at the activity level, with an indication of the financing, and is specified for the duration of the project period. Field assessments and assessment missions will be conducted by UNDP, the Ministry of Health (and jointly), to determine the quantity maps for civil works and, therefore, to ensure valid cost estimates as with for similar projects implemented in the past.	
16. Is the Country Office/Regional Hub/Global Project fully recovering the costs involved with project implementation?	3 2 1	

• **3:** The budget fully covers all project costs that are attributable to the project, including programme management and development effectiveness services related to strategic country programme planning, quality assurance, pipeline development, policy advocacy services, finance, procurement, human resources, administration, issuance of contracts, security, travel, assets, general services, information and communications based on full costing in accordance with prevailing UNDP policies (i.e., UPL, LPL.) • **2:** The budget covers significant project costs that are attributable to the project based on prevailing UNDP policies (i.e., UPL, LPL) as relevant. • **1:** The budget does not adequately cover project costs that are attributable to the project, and UNDP is cross-subsidizing the project. *Note: Management Action must be given for a score of 1. The budget must be revised to fully reflect the costs of implementation before the project commences.	**Evidence** According with the PRODOC, the budget was developed to cover all significant costs. Project-specific costs and HR were incorporated into the budget, as required by the nature of the intervention.
EFFECTIVE	
17. Have targeted groups been engaged in the design of the project? • 3: Credible evidence that all targeted groups, prioritising discriminated and marginalized populations that will be involved in or affected by the project, have been actively engaged in the design of the project. The project has an explicit strategy to identify, engage and ensure the meaningful participation of target groups as stakeholders throughout the project, including through monitoring and decision-making (e.g., representation on the project board, inclusion in samples for evaluations, etc.) • 2: Some evidence that key targeted groups have been consulted in the design of the project. • 1: No evidence of engagement with targeted groups during project design.	3 2 1 **Evidence** The Ministry of Health, The Ministry of Public Works, the provincial and district health authorities are the main project stakeholders. The Ministry of Health will assist in technical designing and monitoring of the necessary regulations and permits. The provincial and district health authorities will play a crucial role in the feasibility study and later accompanying the construction works. Program will be implemented in a way that ensures integration and complementarity of the project into ongoing and continuous health service provision.
18. Does the project plan for adaptation and course correction if regular monitoring activities, evaluation, and lesson learned demonstrate there are better approaches to achieve the intended results and/or circumstances change during implementation?	Yes (3) No (1)
19. The gender marker for all project outputs are scored at GEN2 or GEN3, indicating that gender has been fully mainstreamed into all project outputs at a minimum. *Note: Management Action or strong management justification must be given for a score of "no"	Yes (3) No (1) Evidence This is a civil works project - construction and rehabilitation of infrastructure. Due to the nature of the project, the results of the project were defined with the gender marker: GEN1
SUSTAINABILITY & NATIONAL OWNERSHIP	
	3 2 1

20. Have national/regional/global partners led, or proactively engaged in, the design of the project? • **3:** National partners (or regional/global partners for regional and global projects) have full ownership of the project and led the process of the development of the project jointly with UNDP. • **2:** The project has been developed by UNDP in close consultation with national/regional/global partners. • **1:** The project has been developed by UNDP with limited or no engagement with national partners.	**Evidence** The project will support the Government's efforts towards the achievement of the National Strategic Plan, to lead the provision and production of better health services, universally accessible through a decentralized system that favors partnerships. The project was developed in response to a specific request from the Ministry of Health, which was in charge of determining activities to be included and excluded. UNDP supported the cost assessment activities and prepared detailed project documentation. It is a direct implementation modality project.
21. Are key institutions and systems identified, and is there a strategy for strengthening specific/ comprehensive capacities based on capacity assessments conducted? • **3:** The project has a strategy for strengthening specific capacities of national institutions and/or actors based on a completed capacity assessment. This strategy includes an approach to regularly monitor national capacities using clear indicators and rigorous methods of data collection, and adjust the strategy to strengthen national capacities accordingly. • **2:** A capacity assessment has been completed. There are plans to develop a strategy to strengthen specific capacities of national institutions and/or actors based on the results of the capacity assessment. • **1:** Capacity assessments have not been carried out.	3 2 1 **Evidence** The entire project has been developed to support the Ministry of Health capacities and includes a project management unit (PMU) to assist the Ministry of Health PMU. The content of the Project provides an effective base for knowledge sharing and expansion of good practices: - The Executive Projects can be used in future (re)constructions of health facilities. - Knowledge on personnel level will be shared through involvement in the Project implementation cycle, joint monitoring visits in the field and regular progress debriefs. - At the end of the Project, the Project Board will decide on ways to disseminate Lessons Learned and the knowledge products produced.
22. Is there is a clear strategy embedded in the project specifying how the project will use national systems (i.e., procurement, monitoring, evaluations, etc.,) to the extent possible?	Yes (3) No (1)

23. Is there a clear transition arrangement/ phase-out plan developed with key stakeholders in order to sustain or scale up results (including resource mobilisation and communications strategy)?	Yes (3)	No (1)

Social and Environmental Screening Template (2021 SESP Template, Version 1)

The completed template, which constitutes the Social and Environmental Screening Report, must be included as an annex to the Project Document at the design stage. Note: this template will be converted into an online tool. The online version will guide users through the process and will embed relevant guidance.

Project Information

Project Information	
1. Project Title	Health System Strengthening
2. Project Number	00135528
3. Location	Mozambique
4. Project stage	Design
5. Date	May 2021

Part A. Integrating Programming Principles to Strengthen Social and Environmental Sustainability

QUESTION 1: How Does the Project Integrate the Programming Principles in Order to Strengthen Social and Environmental Sustainability?
Briefly describe in the space below how the project mainstreams the human rights-based approach
The project will strengthen the health system to address health issues of the most vulnerable population. The proposed interventions aim at improvement of the medical supply chain management, HIV and COVID 19 treatment services to ensure equity and access to quality health services to men, women and youth.
Briefly describe in the space below how the project is likely to improve gender equality and women's empowerment
A part of the supply chain designated inequity in infrastructure supply chain management – a larger part of HIV health products and reproductive health products apart from condoms are designated for women. Also better supply chain reduce inequality in access, of which normally women suffer more than men. The gender contribution in HIV and for COVID 19 the gender contribution the improving decentralized access because women´s movements are limited by cultural norms and by need to attend to children on a daily basis. Sex-disaggregated data will be collected through routine medical statistics will be ensured and national District health information system (DHIS). An inherent gender related inequity in infrastructure construction labour sector worldwide exists, as significantly more men are employed in the domain. UNDP will encourage contractors to contract more female employees. The decentralized access will increase possibility for women to find testing and treatment near their home. UNDP will work with stakeholders that the design of infrastructure responds to existing cultural norms and privacy requirements.

Briefly describe in the space below how the project mainstreams sustainability and resilience
The project does not directly contribute to environmental sustainability. It will cover environmental sustainability in the framework of building health system infrastructure. However, minimalization of environmental impact is targeted during the feasibility study (assessment phase). The feasibility study will establish adequate specifications for construction and rehabilitation, so that any possible environment impact is mitigated. Environmental impact in the scope of rehabilitation and construction may relate to procedures on economic and sustainable use and filtration of water, safe waste disposal and sewage and the use of environmentally low impact construction materials. Sustainability and Scaling Up: please refer to page 10 of the PRODOC.
Briefly describe in the space below how the project strengthens accountability to stakeholders
The Project is built on the basis of a strong relationship with the Ministry of Health and assumes an excellent coordination with the Government for the achievement of the Project´s objectives. It is essential that interventions by the MoH such as construction permits are undertaken in a harmonized manner, and that there is continuing knowledge transfer between the MoH and the PMU/UNDP during implementation of the Project. Furthermore, the Project design assumes that: • Stakeholders (Ministry of Health, Ministry of Public Works, Hospital Administrators and local authorities) understand and subscribe to the objectives and long terms goals of the Project, and contribute to the achievement of the desired results; • Government will be able to retain all the capacity created with the support of the Project; • Technical expertise with the knowledge and experience required to achieve the desired results is available and can ensure innovative approaches and highly relevant contributions to the process; • Effective coordination between UNDP programmes, with other UN agencies, and development partners ensures a collaborative approach, promotes synergies and avoids duplication of effort. Please refer to page 9 of the PRODOC.

Part B. Identifying and Managing Social and Environmental Risks

QUESTION 2: What are the Potential Social and Environmental Risks? *Note: Complete SESP Attachment 1 before responding to Question 2.*	QUESTION 3: What is the level of significance of the potential social and environmental risks? *Note: Respond to Questions 4 and 5below before proceeding to Question 5*			QUESTION 6: Describe the assessment and management measures for each risk rated Moderate, Substantial or High
Risk Description (broken down by event, cause, impact)	***Impact and Likelihood (1-5)***	***Significance (Low, Moderate Substantial, High)***	***Comments (optional)***	***Description of assessment and management measures for risks rated as Moderate, Substantial or High***
Risk 1: Human rights Capacity of local authorities and duty-bearers might be limited to support and enable results expected within the Project, due to lack of knowledge and skills and their engagement with other priorities at the district level	I = 3 L = 2	**Moderate**	N/A	Close cooperation will be maintained with local authorities; capacity building activities are envisaged to increase adherence and accountability to project results; the schedule of activities is adjusted to ensure the effective and timely implementation of project activities in the project target areas.
Risk 2: Community Health, Safety and Security Potential that construction and renovation works pose the safety risk to local communities, including children (due to possibility of structural collapse, and physical hazard to health and safety of people during operations)	I = 2 L = 1	**Low**	N/A	Applicable policies, laws and regulations related to potential community, health and safety risks will be properly reviewed and taken into account when drafting the works specifications and implementation of all structural and renovation works.
Risk 3 : Pollution Prevention and Resource Efficiency	I = 3 L = 2	**Moderate**	N/A	Environmental considerations will be integrated into specifications and construction design.

Includes activities that require significant consumption of raw materials, energy, and/or water				

QUESTION 4: What is the overall project risk categorization?		
Low Risk	☐	
Moderate Risk	☑	Capacities of the national and sub-national government institutions and duty-bearers might be limited to ensure appropriate function and prioritization of the HSS and immunization activities, thus affecting the quality of service provided.
Substantial Risk	☐	
High Risk	☐	

QUESTION 5: Based on the identified risks and risk categorization, what requirements of the SES are triggered? (check all that apply)				
Question only required for Moderate, Substantial and High Risk projects				
Is assessment required? (check if "yes")	☑			***Status? (completed, planned)***
if yes, indicate overall type and status		☑	Targeted assessment(s)	Planned for each civil works project.
		☐	ESIA (Environmental and Social Impact Assessment)	
		☐	SESA (Strategic Environmental and Social Assessment)	
Are management plans required? (check if "yes)	☑			

	If yes, indicate overall type		☐	Targeted management plans (e.g. Gender Action Plan, Emergency Response Plan, Waste Management Plan, others)	Environmental management Plan
			☐	ESMP (Environmental and Social Management Plan which may include range of targeted plans)	
			☐	ESMF (Environmental and Social Management Framework)	
	Based on identified <u>risks</u>, which Principles/Project-level Standards triggered?		**Comments (not required)**		
	Overarching Principle: Leave No One Behind				
	Human Rights	☑	Capacities of the national and sub-national government institutions and duty-bearers might be limited to ensure project results (require regular monitoring and timely mitigation measures).		
	Gender Equality and Women's Empowerment	☐			
	Accountability	☐			
	1. Biodiversity Conservation and Sustainable Natural Resource Management	☐			
	2. Climate Change and Disaster Risks	☐			
	3. Community Health, Safety and Security	☑	High risk associated with COVID-19 emergency constraints, Mitigation measures are constantly updated and reported to MoH through the mechanisms described in the Monitoring And Evaluation Plan (please refer to page 14 of the PRODOC).		

	4. Cultural Heritage	☐	
	5. Displacement and Resettlement	☐	
	6. Indigenous Peoples	☐	
	7. Labour and Working Conditions	☐	
	8. Pollution Prevention and Resource Efficiency	☑	Moderate risk associated with construction and rehabilitation materials, tackling waste management and sewage. Mitigation measures will be made part of the project cycle.

Final Sign Off

Final Screening at the design-stage is not complete until the following signatures are included

Signature	***Date***	***Description***
QA Assessor		UNDP staff member responsible for the project, typically a UNDP Programme Officer. Final signature confirms they have "checked" to ensure that the SESP is adequately conducted.
QA Approver		UNDP senior manager, typically the UNDP Deputy Country Director (DCD), Country Director (CD), Deputy Resident Representative (DRR), or Resident Representative (RR). The QA Approver cannot also be the QA Assessor. Final signature confirms they have "cleared" the SESP prior to submittal to the PAC.
PAC Chair		UNDP chair of the PAC. In some cases PAC Chair may also be the QA Approver. Final signature confirms that the SESP was considered as part of the project appraisal and considered in recommendations of the PAC.

SESP Attachment 1. Social and Environmental Risk Screening Checklist

Checklist Potential Social and Environmental Risks		
INSTRUCTIONS: The risk screening checklist will assist in answering Questions 2-6 of the Screening Template. Answers to the checklist questions help to (1) identify potential risks, (2) determine the overall risk categorization of the project, and (3) determine required level of assessment and management measures. Refer to the SES toolkit for further guidance on addressing screening questions.		
Overarching Principle: Leave No One Behind **Human Rights**		**Answer (Yes/No)**
P.1	Have local communities or individuals raised human rights concerns regarding the project (e.g. during the stakeholder engagement process, grievance processes, public statements)?	NO
P.2	Is there a risk that duty-bearers (e.g. government agencies) do not have the capacity to meet their obligations in the project?	NO
P.3	Is there a risk that rights-holders (e.g. project-affected persons) do not have the capacity to claim their rights?	NO
Would the project potentially involve or lead to:		
P.4	adverse impacts on enjoyment of the human rights (civil, political, economic, social or cultural) of the affected population and particularly of marginalized groups?	NO
P.5	inequitable or discriminatory impacts on affected populations, particularly people living in poverty or marginalized or excluded individuals or groups, including persons with disabilities?[1]	NO
P.6	restrictions in availability, quality of and/or access to resources or basic services, in particular to marginalized individuals or groups, including persons with disabilities?	NO
P.7	exacerbation of conflicts among and/or the risk of violence to project-affected communities and individuals?	NO
Gender Equality and Women's Empowerment		
P.8	Have women's groups/leaders raised gender equality concerns regarding the project, (e.g. during the stakeholder engagement process, grievance processes, public statements)?	NO
Would the project potentially involve or lead to:		
P.9	adverse impacts on gender equality and/or the situation of women and girls?	NO
P.10	reproducing discriminations against women based on gender, especially regarding participation in design and implementation or access to opportunities and benefits?	NO
P.11	limitations on women's ability to use, develop and protect natural resources, taking into account different roles and positions of women and men in accessing environmental goods and services? *For example, activities that could lead to natural resources degradation or depletion in communities who depend on these resources for their livelihoods and well being*	NO
P.12	exacerbation of risks of gender-based violence? *For example, through the influx of workers to a community, changes in community and household power dynamics, increased exposure to unsafe public places and/or transport, etc.*	NO
Sustainability and Resilience: Screening questions regarding risks associated with sustainability and resilience are encompassed by the Standard-specific questions below		

[1] Prohibited grounds of discrimination include race, ethnicity, sex, age, language, disability, sexual orientation, gender identity, religion, political or other opinion, national or social or geographical origin, property, birth or other status including as an indigenous person or as a member of a minority. References to "women and men" or similar is understood to include women and men, boys and girls, and other groups discriminated against based on their gender identities, such as transgender and transsexual people.

Accountability		
Would the project potentially involve or lead to:		
P.13	exclusion of any potentially affected stakeholders, in particular marginalized groups and excluded individuals (including persons with disabilities), from fully participating in decisions that may affect them?	NO
P.14	grievances or objections from potentially affected stakeholders?	NO
P.15	risks of retaliation or reprisals against stakeholders who express concerns or grievances, or who seek to participate in or to obtain information on the project?	NO
Project-Level Standards		
Standard 1: Biodiversity Conservation and Sustainable Natural Resource Management		
Would the project potentially involve or lead to:		
1.1	adverse impacts to habitats (e.g. modified, natural, and critical habitats) and/or ecosystems and ecosystem services? *For example, through habitat loss, conversion or degradation, fragmentation, hydrological changes*	NO
1.2	activities within or adjacent to critical habitats and/or environmentally sensitive areas, including (but not limited to) legally protected areas (e.g. nature reserve, national park), areas proposed for protection, or recognized as such by authoritative sources and/or indigenous peoples or local communities?	NO
1.3	changes to the use of lands and resources that may have adverse impacts on habitats, ecosystems, and/or livelihoods? (Note: if restrictions and/or limitations of access to lands would apply, refer to Standard 5)	NO
1.4	risks to endangered species (e.g. reduction, encroachment on habitat)?	NO
1.5	exacerbation of illegal wildlife trade?	NO
1.6	introduction of invasive alien species?	NO
1.7	adverse impacts on soils?	NO
1.8	harvesting of natural forests, plantation development, or reforestation?	NO
1.9	significant agricultural production?	NO
1.10	animal husbandry or harvesting of fish populations or other aquatic species?	NO
1.11	significant extraction, diversion or containment of surface or ground water? *For example, construction of dams, reservoirs, river basin developments, groundwater extraction*	NO
1.12	handling or utilization of genetically modified organisms/living modified organisms?[2]	NO
1.13	utilization of genetic resources? (e.g. collection and/or harvesting, commercial development)[3]	NO
1.14	adverse transboundary or global environmental concerns?	NO
Standard 2: Climate Change and Disaster Risks		
Would the project potentially involve or lead to:		
2.1	areas subject to hazards such as earthquakes, floods, landslides, severe winds, storm surges, tsunami or volcanic eruptions?	NO
2.2	outputs and outcomes sensitive or vulnerable to potential impacts of climate change or disasters?	NO

[2] See the Convention on Biological Diversity and its Cartagena Protocol on Biosafety.
[3] See the Convention on Biological Diversity and its Nagoya Protocol on access and benefit sharing from use of genetic resources.

	For example, through increased precipitation, drought, temperature, salinity, extreme events, earthquakes	
2.3	increases in vulnerability to climate change impacts or disaster risks now or in the future (also known as maladaptive or negative coping practices)? *For example, changes to land use planning may encourage further development of floodplains, potentially increasing the population's vulnerability to climate change, specifically flooding*	NO
2.4	increases of greenhouse gas emissions, black carbon emissions or other drivers of climate change?	NO
Standard 3: Community Health, Safety and Security		
Would the project potentially involve or lead to:		
3.1	construction and/or infrastructure development (e.g. roads, buildings, dams)? (Note: the GEF does not finance projects that would involve the construction or rehabilitation of large or complex dams)	YES
3.2	air pollution, noise, vibration, traffic, injuries, physical hazards, poor surface water quality due to runoff, erosion, sanitation?	NO
3.3	harm or losses due to failure of structural elements of the project (e.g. collapse of buildings or infrastructure)?	NO
3.4	risks of water-borne or other vector-borne diseases (e.g. temporary breeding habitats), communicable and noncommunicable diseases, nutritional disorders, mental health?	NO
3.5	transport, storage, and use and/or disposal of hazardous or dangerous materials (e.g. explosives, fuel and other chemicals during construction and operation)?	NO
3.6	adverse impacts on ecosystems and ecosystem services relevant to communities' health (e.g. food, surface water purification, natural buffers from flooding)?	NO
3.7	influx of project workers to project areas?	NO
3.8	engagement of security personnel to protect facilities and property or to support project activities?	NO
Standard 4: Cultural Heritage		
Would the project potentially involve or lead to:		
4.1	activities adjacent to or within a Cultural Heritage site?	NO
4.2	significant excavations, demolitions, movement of earth, flooding or other environmental changes?	YES
4.3	adverse impacts to sites, structures, or objects with historical, cultural, artistic, traditional or religious values or intangible forms of culture (e.g. knowledge, innovations, practices)? (Note: projects intended to protect and conserve Cultural Heritage may also have inadvertent adverse impacts)	NO
4.4	alterations to landscapes and natural features with cultural significance?	NO
4.5	utilization of tangible and/or intangible forms (e.g. practices, traditional knowledge) of Cultural Heritage for commercial or other purposes?	NO
Standard 5: Displacement and Resettlement		
Would the project potentially involve or lead to:		
5.1	temporary or permanent and full or partial physical displacement (including people without legally recognizable claims to land)?	NO
5.2	economic displacement (e.g. loss of assets or access to resources due to land acquisition or access restrictions – even in the absence of physical relocation)?	NO

5.3	risk of forced evictions?[4]	NO
5.4	impacts on or changes to land tenure arrangements and/or community based property rights/customary rights to land, territories and/or resources?	NO
Standard 6: Indigenous Peoples		
Would the project potentially involve or lead to:		
6.1	areas where indigenous peoples are present (including project area of influence)?	NO
6.2	activities located on lands and territories claimed by indigenous peoples?	NO
6.3	impacts (positive or negative) to the human rights, lands, natural resources, territories, and traditional livelihoods of indigenous peoples (regardless of whether indigenous peoples possess the legal titles to such areas, whether the project is located within or outside of the lands and territories inhabited by the affected peoples, or whether the indigenous peoples are recognized as indigenous peoples by the country in question)? *If the answer to screening question 6.3 is "yes", then the potential risk impacts are considered significant and the project would be categorized as either Substantial Risk or High Risk*	NO
6.4	the absence of culturally appropriate consultations carried out with the objective of achieving FPIC on matters that may affect the rights and interests, lands, resources, territories and traditional livelihoods of the indigenous peoples concerned?	NO
6.5	the utilization and/or commercial development of natural resources on lands and territories claimed by indigenous peoples?	NO
6.6	forced eviction or the whole or partial physical or economic displacement of indigenous peoples, including through access restrictions to lands, territories, and resources? *Consider, and where appropriate ensure, consistency with the answers under Standard 5 above*	NO
6.7	adverse impacts on the development priorities of indigenous peoples as defined by them?	NO
6.8	risks to the physical and cultural survival of indigenous peoples?	
6.9	impacts on the Cultural Heritage of indigenous peoples, including through the commercialization or use of their traditional knowledge and practices? *Consider, and where appropriate ensure, consistency with the answers under Standard 4 above.*	NO
Standard 7: Labour and Working Conditions		
Would the project potentially involve or lead to: (note: applies to project and contractor workers)		NO
7.1	working conditions that do not meet national labour laws and international commitments?	NO
7.2	working conditions that may deny freedom of association and collective bargaining?	NO
7.3	use of child labour?	NO
7.4	use of forced labour?	NO
7.5	discriminatory working conditions and/or lack of equal opportunity?	NO
7.6	occupational health and safety risks due to physical, chemical, biological and psychosocial hazards (including violence and harassment) throughout the project life-cycle?	NO
Standard 8: Pollution Prevention and Resource Efficiency		
Would the project potentially involve or lead to:		

[4] Forced eviction is defined here as the permanent or temporary removal against their will of individuals, families or communities from the homes and/or land which they occupy, without the provision of, and access to, appropriate forms of legal or other protection. Forced evictions constitute gross violations of a range of internationally recognized human rights.

8.1	the release of pollutants to the environment due to routine or non-routine circumstances with the potential for adverse local, regional, and/or transboundary impacts?	NO
8.2	the generation of waste (both hazardous and non-hazardous)?	NO
8.3	the manufacture, trade, release, and/or use of hazardous materials and/or chemicals?	
8.4	the use of chemicals or materials subject to international bans or phase-outs? *For example, DDT, PCBs and other chemicals listed in international conventions such as the Montreal Protocol, Minamata Convention, Basel Convention, Rotterdam Convention, Stockholm Convention*	NO
8.5	the application of pesticides that may have a negative effect on the environment or human health?	NO
8.6	significant consumption of raw materials, energy, and/or water?	YES

RISK LOG

Project Title: "Health System Strengthening" (NFM3)	Award ID: 00135528	Date of last update: 12 May 2021
		Updated by: Ângela Marques

#	Description	Date Identified	Last Update	Type	Impact (I) & Probability (P)	Mitigation measures	Status – May 2021
1	Political situation deteriorates and the government is not able to adhere to the implementation of long-term health strategies due to turnover of key decision makers and changes in priorities.	November 2018	May 2021	Political	I = High P = High	The MoH will adjust the implementation plan of the long-term health strategies (at targets if necessary) to compensate unexpected delays and move toward attainment of development goals. The MoH in collaboration with partners will increase the visibility of public health strategies securing understanding and support from legislative and other branches of executive government to minimize a possible negative impact of turnover of key decision makers and associated revision of health sector priorities. Health Partners Group (HPG) supporting the SWAp revision. UNDP invited to be part of the National Health Observatory, created for the "Analysis of Scenarios, Effects and Measures for the Control of Epidemics" as a way to face the demands emanating from COVID-19 and with the aim of informing health options, practices and policies in the broadest sense against the disease, other endemics and health services. UNDP will follow up with National Health Institute (INS) to understand how this Observatory can be again functional.	The conflict in North of the country is impacting on the country and in particular on the MoH priorities for Emergency Response. UNDP Senior Management is meeting with the Minister of Health to discuss the partnership and inform that UNDP has open a sub-office in Pemba, Cabo Delgado. UNDP Health Programme Manager attending the HPG high level meetings and informed on trends.

#	Description	Date Identified	Last Update	Type	Impact (I) & Probability (P)	Mitigation measures	Status – May 2021
2	Socio-economic situation deteriorates and financial barriers impede access to and utilization of health services.	November 2018	May 2021	Political	I = 5 P = 5	In context of COVID-19 response, the MoH together with development partners leaded by HPG are introducing alternative financing mechanisms and will reallocate available funding to high priority public interventions such as HIV/AIDS, TB and Malaria to mitigate this risk.	UNDP Mozambique requested by donors to implement decentralized interventions 2021-2022 to reach out to target populations in remote areas.
3	Fiscal space shrinks limiting the state's ability to take over fully operational costs and sustain critical health services.	November 2018	May 2021	Political	I = 5 P = 5	In context of COVID-19 response, the MoH together with development partners leaded by HPG are re-prioritizing interventions under these objectives to match resource requirements for the operation with available internal and external funding resources. The Global Fund availed guidelines for flexibilities for programming and eligibility of costs that can be repurposed to emergency response due to COVID-19 pandemic.	UNDP though HSS Programme accepted the MoH requests for support and remains positioned as implementer and technical partner to step up the fight against the spread of COVID-19 in Mozambique.
4	Government internal review and approval procedures could cause delay in the Project implementation.	November 2018	May 2021	Political	I = 5 P = 5	UNDP Senior Management is meeting with the Minister of Health to discuss if Director of Cooperation and Planning may take lead role in signatures when the Secretary Permanent is absent.	Sudden change in the MoH decision makers, namely Secretary Permanent, but as well, Chief of Department of Infrastructure.
5	Contractors selected through public procurement mechanism to construct new and rehabilitate existing health infrastructure fail	2020	May 2021	Operational / Engineering Management	I = 5 P = 3	Fiduciary risk assessment of potential vendors conducted prior to contracting (due diligence and post qualification procedures) and strict mechanisms of financial accountability and operational monitoring and evaluation incorporated in the service contracts to minimize misappropriation of funds.	Engineering Company in place to supervise the quality and ensure timely progress of the civil works. UNDP Engineers, in joint missions with MoH, will also

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	to achieve the value-for-money objective or do not use some portion of funds for intended purpose or are not able to perform as expected. Contractors failure in meeting contractual obligations / deliverables.					Each UNDP Engineer responsible for the contract management of respectively civil works projects, to ensure continuous monitoring of the construction and engineering companies. A Senior Civil Engineer to ensure Engineering Management as per UNDP construction policy is a position approved with new agreement and is being recruited. UNDP is seeking alternatives to co-fund the position to elevate to TA level instead of IPSA, allowing the Engineer to be granted UN Laissez-passer and be able to have Visa and work based in Mozambique as this is core position for the Portfolio.	conduct site visit checks to monitor progress and verify the quality of workmanship. These missions can be conditioned by the restrictions of State of Emergency due to pandemic COVID-19 and to the conflict situation in North of country that affects on Visa not being granted conducting to re-shape of interventions to home-base.
6	Land Ownership Legal documents (DUAT) and Licencing requirements – construction and environmental licenses.	November 2018	May 2021	Legal	I = 5 P = 5	UNDP offered to provide technical support to the MoH, both central and provincial level, to guide on the steps to obtain the different licenses and in filling in the necessary forms.	Focal points from CMAM, INS and from MoH Infrastructure Department supporting the collection of the DUAT for all infrastructures and mediating the communication with the local authorities at the different levels. INS managing exemption of Environmental License in some sites, in context of COVID-19 Emergency context.

#	Description	Date Identified	Last Update	Type	Impact (I) & Probability (P)	Mitigation measures	Status – May 2021
7	Epidemiological situation deteriorates (out-breaks of communicable diseases) due to the climate change related threats and a need arises to reallocate scarce resources urgently.	November 2018	May 2021	Environmental / Public Health	I = 5 P = 4	The MoH is applying to in-country and international donors to provide emergency support for the management of outbreak(s) that have coming out of recurrent cyclones and conflict in the North Mozambique.	UNDP leading the Early Recovery Facility initiative after cyclone IDAI and recently established another sub-office in Cabo Delgado to support in preparedness and response to the War. Recommendations to increase resilience and to ensure safe construction being introduced in the specifications of tender documents of the infrastructures funded by The Global Fund and other donors of Health Portfolio. UNDP though HSS Programme accepted the MoH requests for support in community infrastructure in Cabo Delgado, awaiting funding confirmation.
8	Coverage of hard-to-reach population in some parts of Mozambique can be impeded due to violent, terrorist activities, unusually harsh climatic conditions or natural disasters.	November 2018	May 2021	Environmental / Public Health	I = 5 P = 4	The conflict in North of the country is impacting on the country and in particular on the MoH priorities for Emergency Response. Intervention in Cabo Delgado is being evaluated by UNDP Mozambique due to the humanitarian situation resulting from the increase of terrorist attacks; HSS Programme positioned to contribute on Health Sector.	UNDP Senior Management is meeting with the Minister of Health to discuss the partnership and inform that UNDP has open a sub-office in Pemba, Cabo Delgado. Intervention in Cabo Delgado is being evaluated

#	Description	Date Identified	Last Update	Type	Impact (I) & Probability (P)	Mitigation measures	Status – May 2021
							by UNDP Mozambique due to the humanitarian situation resulting from the increase of terrorist attacks; HSS Programme positioned to contribute on Health Sector.
9	Civil Works specifications incomplete leading to large number of variations. Low quality BoQ and drawings allowing for contractor interpretation.	January 2021	May 2021	Operational / Engineering Management	I = 5 P = 3	Combined on-site evaluations by joint teams (UNDP local recruited senior civil engineer and MoH). Final specifications revised by a UNDP independent Civil Engineer, internationally recruited through UNDP Procurement Support Unit (PSM) roster managed by Global Fund Health Implementation Support Team (GF-HIST) in Copenhagen. Technical discussions hold with MoH and partners. Periodic updates presented to MoH and in Project Board meeting. UNDP introduced innovations in the project type - Validation secured from the MoH.	A Senior Civil Engineer to ensure Engineering Management as per UNDP construction policy is a position approved with new agreement and is being recruited. UNDP is seeking alternatives to co-fund the position to elevate to TA level instead of IPSA, allowing the Engineer to be granted UN Laissez-passer and be able to have Visa and work based in Mozambique as this is core position for the Portfolio.
10	Not enough dissemination of the bidding documents, hence, not sufficient bids received. Short posting timeline *versus* complexity of civil works.	April 2019	May 2021	Operational	I = 5 P = 2	Scope of communication expanded to MoH at central, district and provincial levels. Advertisements in national newspaper with large coverage, after advocacy succeed with donor to support advertisement costs.	UNDP monitor daily the number of bids received and extended the submission deadline for TENDERS as necessary and/or upon request from the bidding companies.

#	Description	Date Identified	Last Update	Type	Impact (I) & Probability (P)	Mitigation measures	Status – May 2021
						National association of construction notified of tender processes information. Project Board members notified of tender processes information.	
11	Gap in core positions of UNDP Operations and Engineering Management. Insufficient number of HR for project implementation contracted at UNDP. Engineers not sufficiently on site for monitoring.	April 2019	May 2021	Operational / HR	I = 5 P = 4	○ Maintained key technical personnel familiar with Project, example, the local recruited civil engineer and architect, through national individual consultant contracts; ○ Project Support Unit recruitments fast tracked for the IUNV and national positions; ○ Consultants from UNDP roster engaged as necessary. ○ Project Support Unit complete and being reinforced in core positions, as per new agreement budget approval. ○ UNDP advancing internal resources to keep the team in place with salaries and to fast track recruitment processes while the new financing agreement is signed.	Restrictions imposed by State of Emergency due to COVID-19 threat conditioning International recruitments for new PMU positions along with Visa not being granted unless the employer has UN Laissez-Passer.
12	Delays in the preparation of the new financing agreement 2021-2023 impacts on HR management, interventions planning and activities implementation.	January 2020	May 2021	Programme	I = 5 P = 5	UNDP advancing internal resources to keep the team in place with salaries and to fast track recruitment processes while the new financing agreement is signed. Implementation slowed down in rhythm but never stopped.	Follow up with PR MoH PMU and GFATM continuously ongoing.

ANNEX IV

Project Board Terms of Reference

General information about the Project

Resilient System for Health is one of the main objectives of any healthcare system in any government. There are about 1651 health facilities registered in public sector database (SARA, 2018). However, there is still a chronic issue of access to quality health services with quality health products. Private sector is not well developed in healthcare service delivery and is mostly confined to pharmacies and dentists in the capital city and some major urban settings.

This Project document is prepared to reflect the details of UNDP support as an Implementing Partner to the Government of Mozambique through the Ministry of Health (MoH), within the newly approved 3-year (2021-2023) Global Fund HIV Grant to Mozambique. The Health System Strengthening interventions align with the UNDP signature solution on poverty as it contributes to improve access to healthcare. UNDP Mozambique through a Financing Agreement (FA) modality will continue to work with the MoH, the Principal Recipient (PR), along with Central Medical Stores (CMAM), National Health Institute (INS) and Department of Infrastructure and Hospital Equipment (DIEH).

The Project is supporting the Government of Mozambique, through the Ministry of Health, in implementing the National Strategies towards achieving:

1. the mission of the National Strategic Plan for Pharmaceutical Logistics and its future integration with the Extended Vaccination Programme (PAV). The Project works to support the MoH in implementing National Logistics Pharmaceutical Plan (PELF) for establishing Medical Warehouses fully equipped including with Incinerators in strategic geographical areas that will ensure medical and non-medical products stocks available in quantity and quality necessary to be distributed through optimized and safer routes to reach the last mile and be accessible to all citizens at all time.
2. the mission of National Emergency Plan for preparedness and response to pandemic COVID-19, through strengthen laboratory capacity for decentralized testing, ensuring adequate and equipped health infrastructure, including through rehabilitation and installation of prefabricated modules. This intervention increases the diagnostic efficiency at sub-national level, aiming at timely providing the citizens with their right to access quality services of treatment and care.

The long-term objective of the Project is to strengthen the national health system by reinforcing the medical supply chain system management and strengthen decentralized laboratory capacity.

This systemic approach is facilitating the current decentralization process of the health system promoted by the government aiming at better and timely delivery of medicines, treatments and healthcare to people at local level. In fact, a key strategy of the Project is implementation at decentralized levels to reach vulnerable populations and to decrease geographical inequalities - contributing to the strategic objectives the National Strategic Health Plan (PESS).

The Project will be implemented by UNDP in Direct Implementation Modality (DIM), while ensuring a participatory approach that involves the counterpart, local authorities and representatives of main

stakeholders in program cycle during the different phases and harnesses the existing strong collaboration with the Ministry of Health through all implementation.

The Theory of Change underlying the intervention fully acknowledges the contribution that the Project will provide towards the overall objective of strengthening the national health system and is framed as follows:

Component 1: Support to PELF implementation and Waste management

It is important to increase storage and distribution capacities for drugs and consumables accompanying the treatment expansion, current storage space and conditions requiring further investment to strengthening quality assurance mechanisms, including the program to combat counterfeit and low-quality medicines.

Attention will be given to:

- Output 1: Improved safety, security and storage conditions of medicines, vaccines and other health products at sub-national level – through:
  - Rehabilitation of the Administrative building, including Demolition and Construction with Expansion up to more than 5200 pallet capacity for Medical Warehouse in Beira city, Sofala province, including Environmental Licensing.
  - Procurement, Installation and Training of Beira Medical Warehouse equipment: Storage Structure and Binning, Handling equipment

- Output 2: Improved systems for adequate waste management – through:
  - Procurement and installation of mobile and containerized incinerator(s) in country Regions capable of running on renewable and sustainable fuel that mid-long term will reduce significantly the maintenance and management costs of the respective Medical Warehouses.

Component 2: COVID-19 emergency response

Since the beginning of the COVID-19 crisis, UNDP has aligned with the efforts of the Government of Mozambique to protect the lives and the livelihoods of its citizens, joining in on the wider United Nations response plan. Within the framework of the current Project, through the partnership with the Global Fund and the Government, National Health Institute (INS), UNDP plans to continue strengthening laboratory capacity for decentralized testing, ensuring adequate and equipped health infrastructure, including through rehabilitation and installation of prefabricated modules.
Attention will be given to:

- Output 3: Strengthened Laboratory capacity for decentralized COVID-19 testing – through:
  - Provision of additional civil works in the Laboratories of Nampula and Zambezia to address biosafety requirements as requested by INS
  - Establishment of three (3) new Laboratories in prefabricated infrastructure in Gaza, Niassa and Manica

- Installation of Generators in the Laboratories of Beira, Tete, Inhambane, Gaza, Niassa and Manica
- Equipment/Furniture of the Laboratories of Tete, Inhambane, Gaza, Niassa, Manica Nampula and Zambezia
- Site evaluation and development of the Executive Projects for the renovations of the three (3) Regional Observatory Centres of INS – Maputo, Nampula and Beira

Project Board/Steering Committee: structure and membership

The Project Board (PB) makes a central element of the Project and is aimed to provide overall guidance and strategic direction to the project, including development, periodic revision, and implementation of the project strategy, and adaptation of global policies and best practices to country circumstances. The PB carries out monitoring and progress assessment of the Project activity and contributes to establishing mechanisms for the Project sustainability in Mozambique. The PB is responsible for ensuring and monitoring of project technical and substantive quality.

The PB will serve as a platform for the major stakeholders of the project to discuss the overall progress of the project and make strategic decisions and recommendations to be implemented by the project team.

The PB will consist of senior staff of UNDP, Ministry of Health, Ministry of Public Works, representatives of relevant government agencies representing interests of beneficiaries, and donor organizations.

Although this is a DIM project, as per current practice the PB meetings are co-chaired by UNDP Deputy Resident Representative and by the Permanent Secretary of the MoH.

The responsibilities of Co-Chairs include:

1. Organization of the Project Board's meetings and invitation of participants
2. Conducting meetings and encouraging all members for equal participation in discussions and evaluation of project
3. Maintaining the meeting procedures as per principles of transparency and efficiency
4. Approving of the Project Board's resolutions and sign Committee minutes

Regular members of Project Board include representatives/senior staff of the following entities:

- Ministry of Health of the Republic of Mozambique: DPC, DAF, DNSP, CMAM, INS, DIEH, Saúde Ambiental – the owner and end-user of the health facilities in this project document. MoH provides inputs into the technical specifications of the equipment and designs of the construction.
- Ministry of Public Works, Housing and Water Resources – is the Government ministry with mandate with planning, building and ensuring effective investment of the public works.
- UN agencies: WHO - to provide technical support to the equipment specifications in line with specific guidance for establishment of health infrastructures and namely in context of COVID-19.

Due to the complexity and inclusiveness of the Project activities, the PB may also invite to the meetings independent representatives and technical experts from other governmental organization, UN agencies, local civil society organizations and international partner organizations, depending on the specific objectives and topic discussed at the specific PB's meeting. To support the Project Board, a technical working group was created as UNDP/ MoH civil works knowledge exchange group. The group can make

technical suggestions and jointly decision making. The suggestions that carry financial impact will need an approval by the PB.

Role and main functions of the Project Board

The PB will act as the coordination and management mechanism for the project. Its major role is to provide strategic oversight and direction of the programme, in order to ensure that it retains strategic focus, and delivers the agreed benefits. It will:

1. Make strategic decisions and provide guidance to senior management of implementing agency
2. Review and approve a consolidated summary annual work plan prepared by the project implementing agency, ensuring that it is focused and consistent with deliverables set out in the Project Document
3. Receive and if necessary, approve progress reports against the work plans and take strategic decisions on how to address any major challenges brought to the PB's attention
4. Monitor progress and impact of any wider issues - e.g. sector reform and other legislative changes, financial situation, programmes by other partners - that might impact upon the project and ensure that these are reflected as necessary within the project.
5. Consider and approve any substantive changes in the action plan or budget of the project upon submission of a solid justification by implementing agencies, should this be necessary
6. Represent, as necessary, the interests of the project in high level government and development partners' discussions

Responsibilities of the PB members

Each member of the PB should have a possibility to carry out the following functions:

1. Get acquainted with the concepts of the project, progress reports and annual work plans in advance, before the PB's meeting is held
2. Consider the progress of the project against the targets set in the project logframe, provide remarks and comments as to reports and work plans
3. Participate in monitoring of the execution of the Project in the field
4. Participate in monitoring and evaluation of the activities implemented by UN agencies and the entire programme as a whole
5. Participate in fact-finding visits to potential beneficiaries
6. If necessary, participate in training and capacity building exercises that the project holds for potential beneficiaries

Requirements to the work and representatives to the PB

The members of the PB should take all required measures to ensure full objectivity of the PB decision, both actual and formal (visible), and should avoid conflicts of interest or excessive influence. The representatives to the PB are obliged to ensure objectivity in the decision-making process using a principle of consensus, to exclude questions of personal character and conflict of interests as well as possible external influences.

In a case where a representative to the PB has any financial interest in the project or a conflict of interests with the project's vendors and contractors, s/he is obliged to inform the members of the PB well in advance and abstain from participation in the discussion even if s/he is not an executor under the project.

Financing

Members of the PB will fulfil the duties on a voluntary no-pay basis, without a financial compensation.

A compensation of expenses related to projects monitoring and evaluation and other Project related activities can be carried out upon submission of all confirming documents, according to the UN procedures and standards, and should be approved prior to expenses are made.

Steering Committee's meetings

The PB will meet semi-annually (semestral basis) to ensure coherence, review progress, adjust programming and endorse joint annual work plans.

The minutes of the PB will be taken by the assigned Project responsible staff. UNDP office in Mozambique will ensure that discussions and decisions taken at the PB are complementary and well communicated to all stakeholders and partners.

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Although this is a DIM project, as per current practice the PB meetings are co-chaired by UNDP Deputy Resident Representative and by the Permanent Secretary of the MoH.

The responsibilities of Co-Chairs include:

1. Organization of the Project Board's meetings and invitation of participants
2. Conducting meetings and encouraging all members for equal participation in discussions and evaluation of project
3. Maintaining the meeting procedures as per principles of transparency and efficiency
4. Approving of the Project Board's resolutions and sign Committee minutes

Regular members of Project Board include representatives/senior staff of the following entities:

- Ministry of Health of the Republic of Mozambique: DPC, DAF, DNSP, CMAM, INS, DIEH, Saúde Ambiental – the owner and end-user of the health facilities in this project document. MoH provides inputs into the technical specifications of the equipment and designs of the construction.
- Ministry of Public Works, Housing and Water Resources – is the Government ministry with mandate with planning, building and ensuring effective investment of the public works.
- UN agencies: WHO - to provide technical support to the equipment specifications in line with specific guidance for establishment of health infrastructures and namely in context of COVID-19.

Due to the complexity and inclusiveness of the Project activities, the PB may also invite to the meetings independent representatives and technical experts from other governmental organization, UN agencies, local civil society organizations and international partner organizations, depending on the specific objectives and topic discussed at the specific PB's meeting. To support the Project Board, a technical working group was created as UNDP/ MoH civil works knowledge exchange group. The group can make

technical suggestions and jointly decision making. The suggestions that carry financial impact will need an approval by the PB.

Role and main functions of the Project Board

The PB will act as the coordination and management mechanism for the project. Its major role is to provide strategic oversight and direction of the programme, in order to ensure that it retains strategic focus, and delivers the agreed benefits. It will:

1. Make strategic decisions and provide guidance to senior management of implementing agency
2. Review and approve a consolidated summary annual work plan prepared by the project implementing agency, ensuring that it is focused and consistent with deliverables set out in the Project Document
3. Receive and if necessary, approve progress reports against the work plans and take strategic decisions on how to address any major challenges brought to the PB's attention
4. Monitor progress and impact of any wider issues - e.g. sector reform and other legislative changes, financial situation, programmes by other partners - that might impact upon the project and ensure that these are reflected as necessary within the project.
5. Consider and approve any substantive changes in the action plan or budget of the project upon submission of a solid justification by implementing agencies, should this be necessary
6. Represent, as necessary, the interests of the project in high level government and development partners' discussions

Responsibilities of the PB members

Each member of the PB should have a possibility to carry out the following functions:

1. Get acquainted with the concepts of the project, progress reports and annual work plans in advance, before the PB's meeting is held
2. Consider the progress of the project against the targets set in the project logframe, provide remarks and comments as to reports and work plans
3. Participate in monitoring of the execution of the Project in the field
4. Participate in monitoring and evaluation of the activities implemented by UN agencies and the entire programme as a whole
5. Participate in fact-finding visits to potential beneficiaries
6. If necessary, participate in training and capacity building exercises that the project holds for potential beneficiaries

Requirements to the work and representatives to the PB

The members of the PB should take all required measures to ensure full objectivity of the PB decision, both actual and formal (visible), and should avoid conflicts of interest or excessive influence. The representatives to the PB are obliged to ensure objectivity in the decision-making process using a principle of consensus, to exclude questions of personal character and conflict of interests as well as possible external influences.

In a case where a representative to the PB has any financial interest in the project or a conflict of interests with the project's vendors and contractors, s/he is obliged to inform the members of the PB well in advance and abstain from participation in the discussion even if s/he is not an executor under the project.

Financing

Members of the PB will fulfil the duties on a voluntary no-pay basis, without a financial compensation.

A compensation of expenses related to projects monitoring and evaluation and other Project related activities can be carried out upon submission of all confirming documents, according to the UN procedures and standards, and should be approved prior to expenses are made.

Steering Committee's meetings

The PB will meet semi-annually (semestral basis) to ensure coherence, review progress, adjust programming and endorse joint annual work plans.

The minutes of the PB will be taken by the assigned Project responsible staff. UNDP office in Mozambique will ensure that discussions and decisions taken at the PB are complementary and well communicated to all stakeholders and partners.